



Tort Law (9th edn)

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p. 247 9. Causation and remoteness of damage

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Abstract

This chapter discusses the final ‘hurdle’ for the claimant to overcome in the tort of negligence—causation. The claimant must prove that their injuries were caused by the defendant’s actions in both fact and law. To establish cause in fact, the claimant must show, on the balance of probabilities, that the defendant’s breach caused their harm. Tests for cause in law encompass a remoteness test (which involves establishing whether the damage that occurred was foreseeable to the defendant at the time of the negligence). It is the type of harm that must be foreseeable, not its extent. The last part of the test is to ask whether any intervening acts (acts that occurred after the defendant’s breach) broke the chain of causation. If so, the defendant will not be liable.

Keywords: tort law, causation, negligence, injuries, liability, intervening act, material increase in risk, material contribution to harm

Problem question

Read this problem question carefully and keep it in mind while you are working through the chapter that follows. At the end of the chapter you will be able to apply what you have learnt to the problem question and advise the relevant parties.

Stefaan and Gavin spend the evening drinking in the pub. Stefaan offers Gavin a lift home in his car, assuring Gavin that this will be fine as he is ‘probably only just over the limit’. Driving home, Stefaan swerves to avoid a fox and crashes the car. The paramedics who arrive at the scene find that Gavin has broken his arm but otherwise only has minor cuts and bruises. Gavin is taken to hospital to be checked by a doctor.

At the hospital Gavin is seen by Cheluchi, the doctor on duty. Cheluchi disagrees with the paramedics' opinion and, deciding Gavin's arm is not broken but only sprained, puts it in a sling, without setting it in a cast. As she was so busy that evening, she decided not to bother sending him for an X-ray first. Gavin returns to hospital the following month with pain in his arm. It transpires that his arm was in fact broken and, because it was not set in the proper cast, the bones have fused together wrongly, resulting in a permanent disability. An expert witness says that there was a chance this might have happened anyway, even if Cheluchi had not been negligent. Gavin has to have an operation to try and re-set the bones, but this will not improve his arm to the condition it was in before the accident.

A week later Gavin is knocked down by a speeding motorist who fails to stop and cannot be traced. His right arm is so badly injured that it must be amputated.

Advise Gavin in relation to the claims in negligence he may bring.

p. 248 9.1 Introduction

Consider the following examples:

→ *Peter goes to hospital complaining of stomach pains after eating some badly prepared fish. A doctor negligently examines him and sends him home. Later, he dies from severe food poisoning. Medical evidence showed that there was nothing the doctor could have done to save him.*

→ *David negligently causes a collision on a motorway. Babatunde, who was driving one of the other vehicles, was dazed by the collision and went to sit on the hard shoulder. A police car, rushing to the scene of the accident, hit and seriously injured Babatunde.*

→ *Sriya had pains in her chest and went to hospital. A doctor gave her a cursory examination and told her to stay in the waiting room, where she later had a heart attack and died. Medical evidence suggested that even if the doctor had treated her properly, she would still have had only a 30 per cent chance of living.*

→ *Bern contracts lung cancer as a result of exposure to asbestos. Medical evidence has proved that the type of cancer he has is caused by a single asbestos fibre entering the lung. Two of Bern's previous employers negligently exposed him to asbestos.*

Once a duty of care and breach are established, claimants must also show that the defendant's breach *caused* their harm. Causation is central to any negligence claim, as it links the defendant's breach to the claimant's harm. The purpose of establishing causation is to allocate responsibility for (the costs of) harm and, to some extent, risks. Causation has two aspects: 'factual' and 'legal'. The first of these is analysed almost mathematically, using a balance of probabilities test (though some exceptions exist, as we will see later) to

establish whether the defendant *in fact* caused the harm. The ‘legal’ aspect of causation (including a test of ‘remoteness’) simply asks whether any of the factual causes should be excluded from being seen as a cause in law. If either causation test ‘fails’, the defendant escapes liability.

Although we use causal language in everyday speech, our meaning is not always clear. The answer to a question about the cause of something often depends on who we ask, or why we ask the question in the first place—different people see causes in different ways, particularly when cause is associated with *responsibility*. Any given event has numerous (often infinite) causes—that is, factors without which it could not have happened. The same is true of a tort or harm. The problem of multiple potential causes runs through causation, leaving the courts having to pin down or ‘find’ the ‘real’ cause from one of several factors. In so doing, two questions must be addressed (see Table 9.1).

- (1) What are the prerequisite conditions for the harm (factual causation)?
- (2) What is the ‘relevant’ or ‘real’ cause of it (legal causation)?¹

Table 9.1 Considering factual and legal causation in negligence

	‘Cause in fact’	‘Cause in law’
The questions being asked	(1) What happened factually in the situation as it occurred?	(1) Even if the breach was the cause in fact of the harm suffered, what is the legal interpretation of the facts—was the harm too remote?
	(2) On the balance of probabilities, did the breach cause the harm alleged (‘but for’ the negligence, would the harm have been suffered)?	(2) Are there any policy or other reasons to justify denying liability?
	(3) If not, have the courts created an exception to the usual rule?	(3) Was there a break in the chain of causation?

p. 249 In law as in life, it is not always possible to pinpoint the single *factual* cause of a harm. A road accident, for example, may have many ‘real’ causes—two drivers may each have been going too fast on a winding country lane in the fog, in which case there are at least three potential causal factors—the speed of each driver and the weather conditions.

Pause for reflection

A stolen and abandoned car is set on fire. Consider this list of potential causes:

- (1) Adam, who stole the car, didn’t tell his parents where he was going.
- (2) Adam’s parents did not check where he was going when he left the house earlier that evening.
- (3) The area lacks recreational facilities for teenagers.

- (4) The police have failed to catch car thieves in the area.
- (5) Adam and the police were involved in a chase; Adam abandoned the car so that he could get away.
- (6) The police failed to notify the Highway Authority that the car was abandoned.
- (7) No one removed the car from the side of the road.
- (8) Bhavna, who set the car on fire, is only 12, but was out unsupervised at night.
- (9) Bhavna's parents don't know that she has left the house.
- (10) Bhavna is sold cigarettes and matches at a local newsagent.
- (11) Bhavna thinks it will be fun to throw lit matches in the abandoned car.

Which of these is/are *the* cause/causes of the harm?

Don't worry if you don't know the 'legal' answer at this stage. The purpose of this exercise is for you to think quickly about which of these facts, if any, you would consider to be causes (and which you would not). Thinking through the issues in this way will help you prepare for the material to follow.

p. 250 As this example helps to illustrate, the process of determining *factual* causation comes down to one of elimination rather than inclusion. How do we go about reducing the list of possible causes? Put another way, are some causes more likely to be legally relevant than others? The answer, it seems, is yes. HLA Hart and AH Honoré suggest that we are more likely to find a ↵ cause in something out of the ordinary or unusual and that where both natural factors and human actions are involved, we will tend to focus on the human actions.² Similarly, positive actions, rather than omissions, are more likely to be considered causative and, when deciding between human actions, we are more likely to focus on voluntary rather than involuntary actions and those that we deem unreasonable rather than reasonable.

In tort law, the question is more sharply focused than simply asking 'what caused the injury or harm?' After all, if this were the question, then any or all the potential reasons given in the list in the previous box could be said to be a 'cause' of the car burning: the car would not have burnt without—or 'but for'—each of the listed facts. Rather, when seeking to establish a claim in negligence, the question is: 'Did the defendant's *negligence* (i.e. their breach of duty) cause this harm?' Or, more precisely, 'Can we say that the act or omission that we have identified as negligent is a "real" cause (it may be one of many) of the harm?' The law approaches this wider question in two stages:

- (1) was the defendant's action/inaction a necessary pre-condition for the harm to occur and, if so;
- (2) is it therefore *the* cause of the harm?

These two stages are known as factual causation (or 'cause in fact') and legal causation (or 'cause in law'), respectively, and, as each contains different principles and tests, will be dealt with separately in this chapter.

9.2 Factual causation—The ‘but for’ test

In determining liability for a breach of duty we must first decide whether the defendant’s wrong *in fact* caused the claimant’s harm. At first glance, this may seem to be a question of common sense. If we ask whether the harm was *a consequence* of the defendant’s breach then we can reach a yes or no answer quite easily. However, in law, the question of causation is actually more difficult, in part due to the fact that there are various *legal* meanings of the word ‘cause’ which differ from its ordinary meanings.

9.2.1 Explaining the test

Sometimes, liability is established simply by showing that the defendant’s actions caused the harm. This is how strict liability systems work. In fault-based systems like negligence, however, this is not enough. We must establish that the defendant’s *breach of duty*, as opposed to simply their *actions*, caused the harm. In other words, the harm suffered by the claimant must be caused by the fact that the defendant’s actions fell below the appropriate standard of care for liability to arise.³

p. 251 The cause in fact question asks whether the defendant’s negligent action or omission was a necessary condition for the harm to occur.⁴ Put differently, if the harm would have occurred *without* the defendant’s carelessness, there is no cause in fact. This is often expressed in the form of a ‘but for’ test: we ask the question ‘But for the defendant’s carelessness (breach), would the claimant have escaped harm?’ If the answer is ‘yes’, the cause in fact is identified: no harm would have come to the claimant without the defendant’s carelessness.

Almost all answers to the factual causation question, however, retain an element of uncertainty. It is probably fair to say that we can never be entirely sure how events would have turned out—for example, whether a particular loss would have been suffered—if the circumstances had been different—for instance, if the defendant *had* exercised reasonable care. Confusingly, we are asking what would have happened in an alternative situation, and yet how can we know, since, by definition, this is not what *did* happen? We deal with this through the ‘burden of proof’. Unlike criminal law, where the burden of proof must be established beyond reasonable doubt, in tort law the claimant must only establish that a breach was, on the balance of probabilities, a necessary condition for the harm occurring. This is a lower standard. To succeed, the claimant need only show that it was *more likely than not* that the defendant’s breach caused their harm. If they cannot establish that the defendant’s negligence made the likely difference between something happening and not happening, then it cannot be a cause of their injuries.

The classic example of the ‘but for’ test is found in *Barnett v Chelsea and Kensington Hospital Management Committee* [1969].

Barnett v Chelsea and Kensington Hospital Management Committee **[1969] 1 QB 428 (QBD)**

A doctor failed to properly examine a man who presented himself at the hospital's emergency department. It transpired that the man was suffering from arsenic poisoning from which he subsequently died. The doctor admitted negligence but said that he had not caused the man's death. *Even if he had have acted properly, the man would have died anyway* as it was too late to do anything to save him. It could not be said that 'but for' the doctor's negligence the man would have survived.

The court held that as the doctor's negligence was not a necessary condition for the man's death, he could not be held liable for it.⁵

In *Nyang v G4S Care & Justice Services Ltd and others* [2013], a similar issue arose. Mr Nyang, detained at an immigration removal centre pending deportation to Gambia, tried to kill himself by running head-first into a concrete wall at speed. He broke his spine and rendered himself tetraplegic. Some of the actions (and inactions) of the detention and medical staff at the centre, particularly in respect of failures to determine his psychological condition, were found to be breaches of duty. However, Lewis J determined that 'but for' the breaches the claimant would have suffered the same harm.⁶ For similar reasoning in different contexts, see *Chubb Fire Ltd v Vicar of Spalding* [2010] (property damage) and *Kirk v Culina Group Ltd* [2024] (death of pedestrian hit by negligently driven heavy goods vehicle).

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9.2.2 Problems with the test

Establishing or disproving cause in fact is not always as easy as in *Barnett*.⁷ We cannot always imagine what would have happened had the defendant not been negligent. Two situations make dealing with factual causation particularly complicated. Here, various policy reasons have led to judicially devised exceptions to the usual 'but for' test.

The first problem is situations where there are multiple potential causes of the harm and a lack of, for example, scientific or forensic knowledge making it too difficult (or even impossible) to establish what would have happened in the absence of negligence. Complex medical situations or accidents involving multiple vehicles are obvious examples of situations where this might lead to an unclear outcome. The second problem, perhaps more contentiously, covers situations where we *can* work out the factual cause, but the result appears unjust and we therefore want to reject it.

This, of course, prompts questions as to exactly *when* such exceptions to the 'but for' test of factual causation are to be recognised. Answers can be found in careful analysis of the difficult and often seemingly contradictory case law and it is to this that we now turn.

9.2.2.1 Multiple potential causes

Where there is more than one potential cause of harm, it becomes hard to establish, in the absence of clear evidence or proof, that any of the potential causes is more likely than any other to be *the* cause. For example, where there are two competing potential causes, each seeming equally plausible, each will only be 50 per cent likely to be the cause of the harm suffered, thus neither would satisfy the ‘but for’ test. Where there are three competing potential causes, each will only be 33.3 per cent likely to be the cause, and so on. A good example is found in *Wilsher v Essex Area Health Authority* [1988].

***Wilsher v Essex Area Health Authority* [1988] 1 AC 1074 (HL)**

Martin Wilsher was born prematurely and cared for in the special care baby unit of a hospital. A doctor was negligent in monitoring his blood. Consequently, Wilsher was, on two separate occasions, given too much oxygen. He developed retrolental fibroplasia (RLF)—an incurable retinal condition—which left him blind in one eye and with seriously impaired vision in the other. Too much oxygen in the blood is a known possible cause of RLF. The defence contended that there were several other ‘innocent’ potential causes, including his premature birth (RLF is a known risk to premature babies). In total, there were five competing causes, each equally probable.

Wilsher’s claim against the negligent doctor succeeded in the lower court and by a majority in the Court of Appeal. However, the House of Lords was not persuaded that, on the balance of probabilities, the doctor’s negligence and not any other factor had caused the harm.

p. 253 ↵ From *Wilsher*, even where there is a clearly negligent act that common sense tells us potentially caused someone’s harm, if there are alternative possible causes of that harm, especially ‘natural’ ones, we cannot establish that ‘but for’ the negligent act, the claimant would not have suffered harm. Consequently, the defendant will escape liability.⁸ The practical problem in multiple potential cause cases is always whether enough evidence can be gathered to ‘prove’ that the defendant’s negligence caused the harm. Variants of the ‘alternatives’ issue arise all the time, but this does not always mean that the claimant will be unsuccessful, as the courts have sometimes deviated from the ‘but for’ test in such situations, as we illustrate next.

Material contribution to harm

In *Bonnington Castings Ltd v Wardlaw* [1956] (predating *Wilsher*), a factory employee contracted pneumoconiosis, a lung condition, from the inhalation of silica dust. He sued his employer in negligence. Some dust inhalation was an inevitable consequence of the work being done (this dust was therefore deemed to be ‘innocent’ dust). However, there was a higher level of dust in the air than there should have been, due to the employer’s negligence in not adequately ventilating the factory (the law lords deemed this ‘guilty’ dust). The question was whether the ‘guilty’ dust made any difference to the claimant’s chance of contracting the disease.

Crucially, medical evidence established that pneumoconiosis was a progressive, or *cumulative* disease—caused by the *build-up* of silica dust in the lungs. It could not be established exactly at what point the disease occurred. As the question was not how much dust was in the lungs or how long it had been there, it could not be established, on the balance of probabilities, whether the ‘guilty’ dust was the cause. That is, it could not be said that ‘but for’ the defendant’s negligence in allowing the ‘guilty’ dust into the air, the defendant would not have suffered the harm.⁹ However, the House of Lords found that because *cumulatively* the ‘innocent’ and ‘guilty’ dust were *more likely* to cause harm, it was enough to show that the defendant’s negligence made a *material* (‘more than negligible’¹⁰) *contribution* to the condition. Effectively, when the ‘but for’ test produced—according to the law lords—unsatisfactory results, they manufactured a way around it.¹¹

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Types of injury: divisible or indivisible?

The distinction between divisible and indivisible injuries is based on the idea that in some conditions/illnesses it is known that greater exposure to the harmful agent in question will likely worsen the condition (a ‘divisible’ injury); whereas in others it cannot be shown that greater exposure will make the condition worse (an ‘indivisible’ injury).¹²

An example of a ‘divisible’ injury would be pneumoconiosis, as in *Bonnington*: the more dust the claimant inhaled, the worse their lungs got. An example often used for ‘indivisible’ injuries is cancer. Cancer either does or does not develop, and does not become more severe if someone is exposed to a greater amount of any agent that caused it.

Following *Bonnington*, where a disease is contracted because of *cumulative* exposure to toxins (divisible), it need only be proved that the negligent part of that exposure would likely materially contribute to the condition, and not that the negligent exposure was *the* probable cause of the condition. In such cases, the claimant will receive full compensation. The effect of this diversion from the ‘but for’ test was somewhat diluted by *Holtby v Brigham & Cowan (Hull) Ltd* [2000].¹³ The claimant was negligently exposed to asbestos by several employers over a period of years, contracting asbestosis (a cumulative condition) as a result. The Court of Appeal ruled that claimants in such situations may only recover damages proportionate to each defendant’s negligence, based on the extent of exposure (time plus intensity) under each negligent employer.¹⁴ The decision can be viewed as significant because the asbestosis was treated as divisible, but the material contribution was made by multiple tortfeasors (unlike in *Bonnington*).

Pause for reflection

Why do you think the Court of Appeal treated asbestosis as divisible in *Holtby*? What would have been the position regarding damages had they not done so? Was the court looking for the ‘fairest’ outcome for both the claimant (who would receive full compensation) *and* the defendants (so all of them

became liable to pay at least something, but none would be liable for the full amount)? See the judgment of Stuart-Smith LJ: 'in my view the court must do the best it can to achieve justice, not only to the claimant but the defendant, and among defendants' (at [20]).

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❖ On the other hand, consider the position of the claimant in this kind of divisible claim who cannot locate one of his previous employers, or who finds that one of them has gone bankrupt. Does the decision to award proportionate damages seem as fair then, particularly when compared with the claimant's position in *Bonnington*, or those where the harm is 'indivisible'? See *Carder v The University of Exeter* [2016], a case illustrating not only the potential perils of apportionment but also the 'justice' that may or may not be done. Recognising the award eventually received by the claimant (just over £1,500) was small, especially when compared to his litigation costs, Lord Dyson MR recommended that similar future claims should be settled early (at [43]).

***Williams v The Bermuda Hospitals Board (Bermuda)* [2016] UKPC 4 (PC)**

Mr Williams went to hospital with severe abdominal pain. The doctor who examined him ordered a CT scan to determine whether he had appendicitis. There was a 4–5-hour delay in performing the scan. Williams's appendix ruptured and he developed sepsis, leading to myocardial ischemia (reduced blood flow to the heart) and his needing life support. He could not prove that 'but for' the hospital's failure to provide adequate diagnosis and treatment, his condition would have been avoided—his claim initially failed for this reason. On appeal, the court found that the negligent diagnosis and subsequent treatment delay had materially contributed to the injury. Bermuda Hospitals Board appealed to the Privy Council.

Lord Toulson applied *Bonnington*, finding the 'parallel with the present case ... obvious' (at [35]), that there had been both 'innocent' (already beginning to occur before the scan) and 'guilty' (attributable to the negligence) sepsis. Crucially, he found that the ischemia resulted from the effect of both. Therefore, the hospital's negligence had materially contributed to the harm.

In *Williams*, the hospital board (with the NHS Litigation Authority intervening) pursued the appeal because it wanted to see 'orthodox legal principles' restored in medical cases (see [3]). However, though non-binding on domestic courts, the Privy Council's judgment indicated that despite some 'distaste' for the idea after *Bailey*, material contribution to harm can apply to medical cases even where the claim relates to an *indivisible* injury.

Some post-*Williams* cases added to the uncertainty in this area. It was applied in the High Court in *John v Central Manchester and Manchester Children's University Hospitals NHS Foundation Trust* [2016], in the context of a brain injury suffered in part because of a fall, compounded by a double delay in diagnosis and treatment

at hospital. Later, in *Davies v Frimley Health NHS Foundation Trust* [2021] it was held that where an *indivisible* injury is caused by a *single tortfeasor*, causation by material contribution is barred. This was approved in *Thorley v Sandwell & West Birmingham Hospitals NHS Trust* [2021]. Soole J noted that although the *Williams* decision (Privy Council) supports the indivisible injury/single tortfeasor idea, he was not bound by it, but he was bound by the Court of Appeal decisions in *Ministry of Defence v AB and others* [2010] and *Heneghan v Manchester Dry Docks Ltd and others* [2016], where such claims were refused. This lack of clarity, he said, meant the law was ‘ripe’ for authoritative review (at [151]).

That may now have occurred, in a recent case decided by the Court of Appeal.

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***Michael Holmes v Poeton Holdings Ltd* [2023] EWCA Civ 1377 (CA)**

Mr Holmes worked for Poeton Holdings for nearly 40 years before retiring for health reasons in 2020. He was diagnosed with Parkinson’s disease, an indivisible disease, in 2014. Holmes claimed Poeton Holdings negligently exposed him to unsafe levels of trichloroethylene—known to be harmful, including by damaging the dopaminergic neurons—this being an accepted cause of Parkinson’s disease—in the earlier part of his employment. In the County Court, HHJ Harrison found that both breach and causation were established and, accordingly, that Poeton Holdings was liable for the full extent of the Parkinson’s disease.

Poeton Holdings appealed on the causation issue, submitting that the wrong legal test for establishing causation in an *indivisible* disease had been applied, and that the judge failed to consider whether Holmes would have developed Parkinson’s disease in any event, so the trichloroethylene exposure made no difference. Further, they contended that the evidence showed only that trichloroethylene increased the risk of contracting Parkinson’s disease; not that trichloroethylene exposure was capable of causing Parkinson’s disease or that Holmes’s exposure had caused the disease.

The Court of Appeal revisited the ‘material contribution’ test in the context of indivisible injury. Having comprehensively reviewed the authorities, the court found that the material contribution test *can* apply to cases of indivisible injury/disease but found that on the expert evidence *in this case* it could not be shown that the defendant’s breach had materially contributed to the claimant developing Parkinson’s disease. Stuart-Smith LJ noted the criticism that the reasoning in *Bailey v Ministry of Defence* [2008] has been subject to. However, he stated:

we are bound in the light of *Bailey* to find that the *Bonnington* ‘material contribution’ principle applies to cases of indivisible injury and that, where the principle applies, the claimant does not have to show that the injury would not have happened but for the tortious exposure for which the defendant is responsible. (at [63])

Thus it seems that we now have authority that the ‘material contribution to harm’ principle applies to both divisible and indivisible injuries, where the evidence supports this.¹⁵ However, we should be cautious: in situations where claimants have difficulty proving factual causation on ‘but for’ grounds, the material contribution option may not be easier (as in *Holmes*); particularly given that it arises in claims at the edge of what medical/scientific evidence can tell us. Even where a material contribution argument is raised, scientific knowledge still may not be able to show that it is more likely than not that the alleged negligence did in fact materially contribute to the harm.¹⁶

p. 257 **Material increase in risk**

The rigidity of the ‘but for’ test was revisited in a different way in *McGhee v National Coal Board* [1973]. The facts were similar to *Bonnington* but with an important difference. Again, scientific evidence could not determine whether ‘innocent’ or ‘guilty’ causes were to blame for the claimant’s condition, a kind of dermatitis related to exposure to brick dust. The claimant encountered ‘innocent’ dust at work, but this same dust became ‘guilty’ *after* work (it stayed on him longer than it should have done, as the employers negligently failed to provide adequate washing facilities). The question was whether the extra time the dust was on the claimant’s skin caused the dermatitis. Unlike pneumoconiosis, dermatitis was *not* a cumulative condition—it could be triggered by a single exposure to the dust—it may, therefore, have occurred from working with ‘innocent’ dust. Thus, the ‘guilty’ dust did not materially contribute *to the harm*, but rather only to the *risk* of harm occurring.

The question became whether the claimant’s condition was caused by his innocent exposure to dust *or* by the ‘guilty’ dust. Only if it could be shown to be the latter would the employer’s negligence be the factual cause of the claimant’s condition. As in *Bonnington*, the evidence could not establish this. However, what could be shown was that the longer the dust lay on someone’s skin, the greater *risk* that person had of contracting dermatitis. The House of Lords held that this was enough—the fact that the *risk* of the claimant developing the condition was materially increased was enough to establish causation.

In *McGhee*, *material increase in risk* was treated as equivalent to *material contribution*. It should be noted, however, that there is still a ‘but for’ calculation involved. To succeed, a claimant must show, *on the balance of probabilities*, that the alleged negligence materially increased the risk of the harm being suffered.

Pause for reflection

The decisions in *Bonnington* and *McGhee* may seem like logical developments to a rigid rule that can produce harsh results. However, look again at the dates of the two cases, and then at *Wilsher*.

Both cases predate (and, importantly, survive) *Wilsher*. In fact, the *McGhee* argument was rejected by the House of Lords in *Wilsher*, which did not think that *McGhee* had established a new test for causation.¹⁷ They said that although there was a materially increased risk of blindness caused by the doctor’s negligence, this was not enough to establish factual causation. There were four other possible

‘innocent’ causes, any of which may have led to the condition. Why was there a difference in this case? Does the fact that there was *more than one* other potential cause make the difference? Or is the decision in *Wilsher* based purely in policy, showing (as is often the case in relation to establishing breach—see **Chapter 8**) undue deference to the medical profession? If so, is the Court of Appeal’s decision in *Bailey v Ministry of Defence* [2008] consistent?¹⁸ For a visual outline of the judicially created exceptions to the normal causation rules, see **Figure 9.1** later.

Multiple potential causes in fact		
Problem: C cannot prove on the balance of probabilities that D was the cause in fact, either because of one or more other D's potential causal contribution, or because of other natural or 'innocent' causes, or because of some fault of C		
Solutions: courts have created some limited exceptions to the usual rules		
Impossible to prove 'but for' (via e.g. scientific or medical evidence (e.g. <i>Wilsher</i>) (9.2.2.1))	Fails on balance of probabilities but result appears unjust (9.2.2.2)	
Cumulative causes/conditions	Non-cumulative causes/conditions	Non-cumulative causes/conditions
Negligent act makes a 'material contribution' to the <i>harm</i> ↓ Bonnington D was therefore liable for it (100%) ↓ However, since <i>Holtby</i>, damages awarded <i>proportionately</i> by D (so D might not end up being liable for 100% of the damages) – i.e. is a 'divisible injury' ↓	Negligent act makes a 'material contribution' to the <i>risk</i> of harm ↓ McGhee ↓ Still results in a 'but for' test: C must show it is more likely than not that the negligent act materially increased the risk ↓	In <i>Fairchild</i>, mesothelioma could have occurred from negligent exposure to asbestos from one of a number of employers. Condition is NOT cumulative ↓ Applied McGhee 'material contribution' to the <i>risk</i> of harm test ↓ All negligent employers held <i>jointly and severally</i> liable (each liable for 100% of the damages) ↓ Barker introduced <i>proportionate</i> damages, but this was overturned <i>in relation to mesothelioma only</i> by section 3 of the Compensation Act 2006 ↓ Fairchild treated as exception to normal rule, distinguishing <i>Wilsher</i>. Its exceptional nature confirmed in <i>Sienkiewicz</i> and <i>MOD v AB and others</i> and most recently <i>Zurich</i> (test appeared only to apply to mesothelioma – but now see <i>Henaghan</i>) ↓
Proportionate damages	Joint and several liability	Mesothelioma: joint and several liability Other injuries: proportionate damages

Figure 9.1 Problems and solutions to multiple potential cause claims

- p. 258 ↵ A key development in this area came with the House of Lords' decision in *Fairchild v Glenhaven Funeral Services* [2002]. However, despite it being a case of 'material increase in risk', this case also fits into another category: that where the court feels the result of using the 'but for' test is an unjust one.

9.2.2.2 'Unjust' results 1: The mesothelioma exception

Fairchild v Glenhaven Funeral Services [2002] UKHL 22 (HL)

A group of claims were made against multiple defendants by three employees who developed mesothelioma (an inevitably fatal type of lung cancer), caused by exposure to asbestos. Two of the claims failed at first instance, as the claimants could not show where the asbestos that caused their mesothelioma had come from.

In relation to the claim in *Fairchild*, however, there was no problem in establishing that the employer negligently allowed the claimant to be exposed to asbestos—the problem was that the employee had been exposed by a series of negligent employers, not just one. As in *McGhee*, the experts agreed that mesothelioma could be caused in a single moment—for example, even by a single asbestos fibre entering the lung—and not necessarily by prolonged exposure.¹⁹ The difficulty was that the claimants could not show *when* that causative exposure occurred, so they could not show, on the balance of probabilities, which of the employers was the factual cause. On that basis, strictly following the 'but for' test, the Court of Appeal dismissed the claim, and the claimant appealed to the House of Lords.

The law lords held that the 'but for' test produced an unjust result. The idea that no defendant would be liable for the claimant's harm, when there was no doubt that the cause had been negligence, sat uncomfortably with them. Accordingly, they resurrected *McGhee*, approving the 'material increase in risk' interpretation and stating that in this *exceptional* type of case (perhaps defined as employment/industry)²⁰ this would be enough to establish factual causation; the defendants were therefore liable.²¹

In finding in *Fairchild* that each defendant had materially increased the risk of the claimant contracting mesothelioma, the House of Lords did not overrule *Wilsher*, which seemed to potentially stand in the way of this interpretation. Instead, they distinguished it, holding that *Wilsher* had been correctly decided on its facts.²² The law lords stressed that *Fairchild* was an *exception* to normal causation principles. The outcome of *Fairchild* was that each negligent employer was held jointly and severally liable. It would therefore be up to the employers who had been brought before the court to seek contributions from others who had not (under s 1(1) of the Civil Liability (Contribution) Act 1978).

Pause for reflection

Does the fact that *Wilsher* and *Fairchild* are both good law have any impact on the type of case the ‘material increase in risk’ test can be applied to in the future? We do not find the explanation that *Wilsher* should be confined to its facts convincing, and would rather have seen a more carefully stated exposition of the policy and other distinguishing arguments behind the decision to depart from what seemed to be binding authority. Was it, for example, the difference between finding employers (who were likely to be insured) liable and the NHS? Did the shorter timescale in *Wilsher* indicate a different outcome—the claimants in *Fairchild* had worked and were exposed to asbestos over a long period of time, followed by the slow onset of their disease and the inevitable nature of their death?

A clearer explanation might be found in *Ministry of Defence v AB and others* [2012], a test case involving claims relating to around 1,000 people harmed (including various cancers) by nuclear testing in the Pacific in the 1950s. Evidence showed there were several potential causes of the cancers other than radiation. While the judge at first instance thought that the higher courts might be happy to extend the *Fairchild* exception to this kind of case ‘as a matter of policy’, the Court of Appeal found ‘no foreseeable possibility that the Supreme Court would be willing to extend the *Fairchild* exception’ in cases where multiple potential causes exist, some of which were impossible to identify.²³ And they were right: in the Supreme Court Lord Phillips observed that, in light of the observations made in *Sienkiewicz v Greif (UK) Ltd* [2011], the Court of Appeal’s conclusion that the *Fairchild* exception would not be extended was correct (at [157]). There is clearly ‘no appetite in the appellate courts for extending the *Fairchild* exception to cases involving diseases other than mesothelioma’.²⁴

In *Barker v Corus UK Ltd* [2006], the House of Lords revisited similar issues, though with the added complication of a period of self-exposure and a new question regarding apportionment of damages.

Barker v Corus UK Ltd [2006] UKHL 20 (HL)

Various claims were made on behalf of people who died from mesothelioma contracted due to exposure to asbestos dust at work. Some exposure was negligent on the part of the employers concerned and the claims were brought against them (those that had not gone bankrupt since the injury occurred). In one claim, the exposure could be clearly broken down to three distinct periods of time: a period when the claimant was working for a now insolvent employer (who therefore could not be sued); a period when working for the defendant; and a period of self-employment. At both first instance and in the Court of Appeal, following *Fairchild*, the defendants were held jointly and severally liable.

↵ However, to reflect the claimant's own negligence during the period of self-employment, contributory negligence (a partial defence, see **Chapter 10**) was found, with damages reduced by 20 per cent to take this into account. The defendants appealed the finding of joint and several liability to the House of Lords, arguing that liability should be proportionately divided to reflect the actual risk that was caused by their negligence (e.g. based on the proportion of time that the claimant was negligently exposed to asbestos by the employer).

A majority of the House of Lords agreed that the damages should be apportioned among the defendants according to their contribution to the increased risk.²⁵ They agreed that the most 'practical' method of apportionment would be based on the amount of time the claimant was negligently exposed to asbestos by each employer, Lord Hoffmann noting that 'allowance may have to be made for the intensity of the exposure and the type of asbestos' (at [48]).

In *Barker*, the House of Lords departed from the idea that the employers, all of whom had contributed by their negligence to an increased risk of the claimant contracting a fatal lung cancer, should be jointly and severally liable. Instead, they preferred the idea that compensation should be based on the period of time (and intensity of exposure, etc) that the claimant worked for the employer and was negligently exposed to asbestos.²⁶ This issue was never raised in *Fairchild*. In practice, this means that, for example, in a situation where an employee was negligently exposed to asbestos by a series of four employers and one of these employers had subsequently gone bankrupt, compensation—but not full compensation—would be divided between the remaining three solvent employers according to the period of time the claimant was employed by them. This means that the claimant cannot fully recover the damages that have been calculated according to their (or their dependants') need. This principle is illustrated in **Table 9.2**.

Table 9.2 Division of damages between a series of employers (following *Barker*)

	Risk increased by negligent defendant	Period of time employed by defendant	Defendant solvent or bankrupt	Proportion of damages paid
Harm suffered by claimant (e.g. mesothelioma)	A	10 years	Solvent	20%
	B	20 years	Bankrupt	None
	C	15 years	Solvent	30%
	D	5 years	Solvent	10%
Total proportion of (full) damages recoverable by claimant				60%

Pause for reflection

In *Fairchild*, the Court of Appeal found none of the defendants liable, as it could not be established, on the balance of probabilities, that any one of them had caused the claimants' harm. They said that the required 'leap over the evidential gap' would not only defy logic but would be 'susceptible of unjust results':

If we were to accede to the claimants' arguments, we would be distorting the law to accommodate the exigencies of a very hard case. We would be yielding to a contention that all those who have suffered injury after being exposed to a risk of that injury from which someone else should have protected them should be able to recover compensation even when they are quite unable to prove who was the culprit. (Brooke LJ at [103])

The House of Lords disagreed. Lord Bingham, dealing with this exact point, explained:

There is a strong policy argument in favour of compensating those who have suffered grave harm, at the expense of their employers who owed them a duty to protect them against that very harm and failed to do so, when the harm can only have been caused by breach of that duty and when science does not permit the victim accurately to attribute, as between several employers, the precise responsibility for the harm he has suffered. I am of the opinion that such injustice as may be involved in imposing liability on a duty-breaking employer in these circumstances is heavily outweighed by the injustice of denying redress to a victim. (at [33])

Which of the opinions do you agree with most? How does your opinion fit with the result of *Barker*?²⁷

Dissenting in *Barker*, Lord Rodger said that redefining the harm to justify apportioning damages between defendants would unfairly shift the risk that defendants had gone bankrupt, or were otherwise untraceable, onto claimants, who had been negligently injured by the defendants in the first place. *Barker*, unsurprisingly, caused great concern among workers who had been negligently exposed to asbestos during their employment.²⁸ These concerns were recognised by Parliament which, at the time, was debating the (then unrelated) Compensation Bill. As a result of the hostility to *Barker*, the Bill was amended to legislatively reverse *Barker*'s effect. Section 3 of the Compensation Act 2006 provides that, where the conditions for applying *Fairchild* are met (i.e. where negligent exposure to asbestos by a series of employers causes mesothelioma, but it cannot be shown which of the employers actually, or most likely, caused the disease), any or all employers will be liable for the whole sum of damages.²⁹ This applies even where other sources of exposure existed, including self-employment or other (non-negligent) sources (though damages may still be reduced for the claimant's own contributory negligence under section 3(3)(b)). It is, of course, open to the liable defendant to seek contributions from others under the principles governing joint and several liability.³⁰

Counterpoint

Had section 3 of the Compensation Act 2006 not been enacted, we would, in our opinion, have been left in an undesirable and contradictory position regarding House of Lords' decisions on asbestos-related claims. In *Fairchild*, the position was that claimants should be allowed to recover *full* damages for the harm (and it must be remembered that mesothelioma is a fatal lung cancer), even though they could not possibly have established which of the defendants was the likely cause. In this sense, the *Fairchild* decision is claimant-centred and is based on the fact that the claimant would receive nothing if the 'material increase in risk' exception to the usual 'but for' rule was not used.

In contrast, *Holtby*, while a good result for the actual claimant in that case, means that in future claimants suffering from asbestosis (as opposed to mesothelioma) may not recover in full if one or more of their previous employers has become insolvent.³¹ This is less claimant friendly. The same can be seen in the House of Lords' decision in *Barker* on the same point regarding mesothelioma.

In *Barker*, Lord Rodger pointed out that while it may seem harsh that a single defendant becomes liable for the total sum of a claimant's damages, purely because his negligence made a material contribution to the risk that the claimant would suffer mesothelioma, this was also settled law. He went on to say that this principle 'is a form of rough justice which the law has not hitherto sought to smooth, preferring instead, as a matter of policy, to place the risk of insolvency of a wrongdoer or his insurer on the other wrongdoers or their insurers' (at [90]). *Barker*, he claimed, meant that this risk must be 'shouldered entirely by the innocent claimant' (at [90]). This does not appear consistent with the policy-orientated decision made by the House of Lords that sometimes wrongdoers should pay even where it cannot be proved that it was *their* wrong that caused the harm. As Lord Rodger found in *Barker*, 'the desirability of the courts, rather than Parliament, throwing this lifeline to wrongdoers and their insurers at the expense of claimants is not obvious' (at [90]). This is particularly true given the fact that a claimant who contracted mesothelioma and was only ever employed by one negligent employer over the course of their working life would be able to recover full damages based on the 'but for' principle. Claimants should not be hindered by the fact that they have worked for *more* than one negligent employer.

↪ Clearly Parliament agreed and section 3 of the Compensation Act 2006 removes this risk from claimants who have contracted or died from mesothelioma.³² What section 3 does not legislate is what would happen if *Fairchild* is applied *outside* a claim for mesothelioma. It seems in that instance that *Barker*'s proportionate liability will still apply.³³

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Watch our video 'Disease, causation and government policy' where we discuss whether there is a continuing case for judicial exceptions to tort principles to be made for some diseases but not others.

Video playback is not supported in this format.

Video 9: Disease, causation and government policy

Because the effect of the interaction of the decisions in *Fairchild* and *Barker* was not immediately clear, a flurry of causation cases based on the idea of material increase in risk emerged. Few succeeded.³⁴ Later, the Supreme Court indicated that the idea that there is a separate claim for material increase in risk *outside* of mesothelioma is wrong.³⁵

Though *Fairchild* will apply in mesothelioma cases where there have been multiple negligent exposures to asbestos, there remained a question about what happens when there has only been a single (negligent) exposure coupled with ‘ordinary’ environmental exposure. The impact of section 3 of the Compensation Act 2006 was called into question in this respect in *Sienkiewicz*.

***Sienkiewicz v Greif (UK) Ltd* [2011] UKSC 10 (SC)**

An action was brought on behalf of Mrs Costello, who died from mesothelioma in 2006. Mrs Costello worked in offices at the defendant’s factory in Ellesmere Port in Cheshire for nearly 20 years until 1984. Her job required her to enter areas of the factory that were contaminated with asbestos. At first instance, this exposure was found to be negligent. However, the judge also found that the claimant had been exposed to low levels of asbestos in the general atmosphere around Ellesmere Port, which might also have been significant enough to have caused mesothelioma.

The defendants argued that where there was only one employer, the correct test was the ‘but for’ test, not the *Fairchild* exception. They contended that the claimant had to prove on the balance of probabilities that the negligent exposure caused the disease and that to do this she would need to establish that the negligent exposure more than doubled the risk.³⁶ The judge accepted this argument, finding also that the doubling of risk could not be established on the facts—the negligent exposure had only increased the risk by 18 per cent. The claimant appealed, arguing that the correct test was whether the (negligent) occupational exposure had *materially* increased the risk: the increase did not have to be twofold, only more than minimal. The Court of Appeal agreed.³⁷ On the issue of apportionment, Smith LJ first acknowledged that the *Fairchild* judges *might* not have thought the exception appropriate where there was only one negligent employer and the other asbestos source was environmental. However, whether this was the case or not, she found that section 3 of the Compensation Act 2006 rendered this irrelevant.³⁸ Lord Clarke thought the case was clearly the type of case considered in *Fairchild*, as interpreted in *Barker* (as the non-occupational exposure could be viewed in a similar way to the period of self-employment in *Barker*). Unanimously, the appeal was allowed.

The defendants appealed to the Supreme Court. The seven-member court considered the ‘*Fairchild* exception’, the effect of section 3 of the Compensation Act 2006, how ‘material’ a material increase in risk has to be (rejecting the so-called ‘doubling of risk’ idea), as well as the importance of epidemiological evidence, eventually dismissing the appeal unanimously.³⁹ Lord Phillips (at [98])–

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[106]) explained that a continuing justification for maintaining the exception is that the gaps in our knowledge as to how mesothelioma is triggered ‘justifies the adoption of the special rule of causation that the House of Lords applied in *Fairchild* and *Barker*’ (at [103]).⁴⁰

This decision tells us that the ‘*Fairchild* exception’ also applies in mesothelioma cases where only *one* defendant is proved to have negligently exposed the victim to asbestos, even though there was a risk of developing the disease from a non-negligent exposure.⁴¹ The Supreme Court found that proof that the injury would not have happened ‘but for’ the defendant’s negligence is unnecessary: a *material increase in risk* is enough. It also confirmed that section 3 only applies when the defendant has committed a tort.⁴² As confirmed in *Zurich* [2015], and taking us back to the question of what exactly ‘cause’ means, we now know that under *Fairchild*, as confirmed in ‘*Trigger Litigation*’ [2012], mesothelioma is ‘caused’ in any period in which exposure occurs which materially increased the risk of contracting it.⁴³ The Court of Appeal has more recently determined that *Fairchild* also applies to lung cancer caused by exposure to asbestos.⁴⁴

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9.2.2.3 ‘Unjust’ results 2: Failure to inform

Outside asbestos-related litigation, the courts have departed from ordinary ‘but for’ causation to secure just results in medical negligence cases involving ‘informed consent’. In *Sidaway v Board of Governors of the Bethlem Royal Hospital* [1985], a surgeon failed to disclose a 1–2 per cent risk of spinal damage to a patient undergoing a spinal operation, which later materialised. In all probability, the House of Lords agreed, he had failed to disclose the risks. However, other experts said they would not have disclosed the risks in the same situation either, so counsel for *Sidaway* tried to invoke the transatlantic informed consent doctrine. The law lords refused to entertain this (Lord Scarman dissenting), declaring the doctrine impractical and stating that the time was not right to substantially change the rules on informing of risks. However, Lord Templeman noted that if *Sidaway* had *asked* about the specific risks, she should have been informed of them.

Chester v Afshar [2005] UKHL 41 (HL)

The claimant developed a serious spinal condition (cauda equina syndrome) following an operation on her spine, which she had been advised to undergo by the defendant. The claimant had reluctantly agreed to the operation. The defendant had not warned her of the small risk that she might develop the condition, even if the operation was performed without negligence. When this risk materialised, the claimant sued in negligence arguing that had she been aware of the risk she would have sought advice on alternatives to surgery and the operation would not have taken place when it did (although it may well have taken place at a later date). The trial judge found that the possibility that the claimant might have consented to surgery in the future was not sufficient to break the causal link between the defendant’s failure to warn and the damage sustained by the claimant. The Court of Appeal agreed.

So did the majority of the House of Lords. They found that the claimant was not able to establish sufficient causation on conventional principles. Because she may well have had the operation later, the defendant's failure to warn neither affected the risk nor was the effective cause of the injury she sustained.⁴⁵ However, the law lords went on to modify these principles. The defendant doctor owed her a duty to advise her of the disadvantages of the treatment he proposed and this was closely connected with her ability to give consent and make an informed choice about whether to have treatment. Since the injury she sustained was within the scope of the defendant's duty to warn and was the result of the risk of which she was entitled to be warned, the defendant's failure to warn the claimant was a breach of this duty *and* the cause of the claimant's injury: 'Her right of autonomy and dignity can and ought to be vindicated by a narrow and modest departure from traditional causation principles' (Lord Steyn at [24]).

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Counterpoint

Note that *Sidaway* was decided at a time when medical paternalism was rife, whereas ***Chester*** came at a time when patient rights were more carefully considered (following the implementation of the Human Rights Act 1998) and, despite not being argued this way, Carole Chester's human rights certainly seem to have been championed (also see *McGlone v Greater Glasgow Health Board* [2011], a Scottish case). That said, ***Chester*** did not get rid of the *breach* question in relation to informed consent, merely sidestepped it. There remained a question about what risks it was legitimate for a doctor to tell or not tell, without falling below the standard of care expected of them. On this, see ***Montgomery v Lanarkshire Health Board* [2015]**, discussed in **section 8.3.2.1** and subsequent cases in this area.

Though the law lords viewed ***Chester*** as a departure from ordinary 'but for' principles, another interpretation is that it represents merely a (less than) straightforward application of the 'but for' test. But for the defendant's negligent advice, Ms Chester would have avoided injury (on the balance of probabilities) as she would either not have had the operation at all or, even if she had had the operation on another day, there would only have been a 1–2 per cent chance of the risk materialising. That is, it was 98–99 per cent unlikely that she would have suffered the injury. Indeed, this had been the view of the Court of Appeal,⁴⁶ and has since been interpreted as the correct view by the High Court of Australia.⁴⁷

In *Correia v University Hospital of North Staffordshire NHS Trust* [2017] the Court of Appeal emphasised that if ***Chester*** is to be relied upon it remains necessary for the claimant to prove that, if correctly warned of the risk, they would have declined or deferred the operation. This was confirmed in *Duce v Worcestershire Acute Hospitals NHS Trust* [2018] and *Diamond v Royal Devon & Exeter NHS Foundation Trust* [2019]. In *Duce*, it was found that the claimant could not establish that she would not have gone ahead with the operation, which had left her with nerve damage, had she been warned of the risks. In *Diamond*, it was established that the claimant would have proceeded with abdominal mesh surgery even if she had been informed of the risks in relation to potential future pregnancies. An example of a case where it was established that, had the claimant

been warned of the risks of her surgery, she would *not* have undergone the operation (which left her paralysed) is *Hassell v Hillingdon Hospitals NHS Foundation Trust* [2018].⁴⁸ In *Khan v Meadows* [2021] it was confirmed that only the risk that it was the duty to be warned against was actionable, not the risk of any additional harms that might materialise.⁴⁹

9.2.2.4 Indeterminate causes

p. 268 What is the situation when there is more than one negligent defendant, but *neither* can be proved to be the cause of the harm suffered? This is what is known as an ‘indeterminate cause’. If it is accepted that mesothelioma, for example, is caused by a single fibre of asbestos entering the lung, then *Fairchild* becomes a case of indeterminate causes, because each ↵ negligent employer was equally likely to have exposed the claimants to the asbestos that caused the disease. Neither satisfies the ‘but for’ test. By contrast, *McGhee*, on which the decision in *Fairchild* rests, is not a case of indeterminate cause, as the dust all came from the same place, albeit some ‘innocent’ and some ‘guilty’.

In cases from other jurisdictions raising the issue of the inability to prove which of two negligent defendants actually caused the harm, courts found another novel way to circumvent the ‘but for’ test. In the US case of *Summers v Tice* [1948] and the similar Canadian case of *Cook v Lewis* [1951], the court shifted the burden of proof to the defendants. In both cases, the issue was which of two negligent hunters fired the shot that hit the claimant (and hence who should be liable). Both had fired their guns at approximately the same moment. Evidence could show only that the shot fired came from one of their two guns—applying the ‘but for’ test the claimant could never win, as neither defendant would be more than 50 per cent likely to have fired the offending shot. Shifting the burden of proof meant that the court asked the defendant to prove that he was *not* responsible—if neither could do this then they should be jointly and severally liable. In *Fairchild*, the House of Lords, discussing the ‘two hunters’ scenario, explicitly rejected the idea of reversing the burden of proof, deeming it artificial: proof was the problem in the first place (Lord Hutton at [110]).

Similar to the ‘two hunters’ problem is *Fitzgerald v Lane* [1987], where a pedestrian crossing a road was hit first by one negligent driver and then almost immediately by another, rendering him tetraplegic. It was impossible to establish which impact actually caused the tetraplegia (and it may well have been both) and applying the ‘but for’ test would therefore have rendered each of the negligent drivers equally (50:50) responsible but, crucially, neither liable for the harm. The Court of Appeal, using the *McGhee* ‘material increase in risk’ principle, found each defendant liable.⁵⁰

Pause for reflection

In another US case, *Sindell v Abbot Laboratories* [1980], the problem of indeterminate causes was resolved differently. Five pharmaceutical companies were all held liable for supplying a drug that caused vaginal cancer in women even though, because the cancer developed a long time after the drugs had been taken, it could not be established by any of the claimants which company supplied the particular drug that caused their harm. Any of the five companies could have been responsible, as the

problem was in the design of the drug itself, rather than a manufacturing error. Had the 'but for' test been applied, none of the companies would have been liable. The court surmounted this problem by adapting the rule from *Summers v Tice* and apportioning liability on the basis of each defendant's market share. Therefore, *all* the companies were liable (and so all had to pay), but the court also decided that the *amount* each should pay should be based on that company's share of the market.

Do you think this is a justifiable decision? Is apportioning the damages to be paid in this way more appropriate when the harm is caused by a product (rather than employers), particularly when all companies posed an equal *risk* of causing vaginal cancer?

p. 269 9.2.2.5 Uncertain actions

So far we have considered cases where it has been difficult to show what has actually happened, thus making the application of the 'but for' test problematic. A different problematic situation arises in circumstances when either the claim being made relates to something that has not been done, but should have been (an omission), or—more precisely—when we cannot be quite *sure* what would or should have been done in a given situation.

In *Bolitho v City and Hackney Health Authority* [1998], it was alleged that a hospital consultant was negligent in failing to respond promptly to her pager, as a result of which the 2-year-old claimant died. In seeking to establish a causal link, the House of Lords had to consider what the consultant *would* have done had she responded. The consultant gave evidence that even if she had attended the child, she would not have intubated him. Expert evidence showed that this was the only course of action that could have saved him at that time. So viewed, on the balance of probabilities, the child would have died whether the consultant responded or not. However, the law lords approached the case differently, finding that it must first be established whether the doctor's failure to intubate would also have been negligent (applying the test from *Bolam v Friern Hospital Management Committee* [1957]). If it was, legal causation was *also* established because it could be said that what the doctor *should* have done, but negligently did not do, was the likely cause of the child's death.⁵¹

9.2.2.6 Loss of chance

There has been an interesting and different response to another type of causation problem, sometimes referred to as 'loss of chance'.

***Hotson v East Berkshire Health Authority* [1987] AC 750 (HL)**

A boy fell from a tree, injuring his hip. In hospital, he was negligently diagnosed: the doctors who saw him failed to notice that vascular necrosis (a condition where blood vessels die) had developed and he was sent home without adequate treatment. Five days later he returned to hospital and his condition was correctly diagnosed, though by this time nothing could be done and paralysis was inevitable. Expert medical evidence indicated that had the initial diagnosis been made correctly, there would have been a 25 per cent chance that the condition could have been prevented. However, five days later, that chance had gone.

The House of Lords found that it could not be shown on the balance of probabilities that Hotson would have avoided the condition had the doctors not been negligent: it was more likely than not that he would have developed the condition even with proper diagnosis. Therefore, his claim failed.

p. 270 *Hotson*, like *Barnett*, perfectly illustrates exactly what a claimant needs to pass the ‘but for’ test: it must be more likely than not that the defendant’s negligent actions caused the damage. Though *Hotson* is ultimately a straightforward application of the ‘but for’ test, the case ↵ raised further interesting issues. Both lower courts accepted an alternative argument (or an alternative definition of the *harm*) that the claimant *could* have established that he had ‘lost a chance’—the 25 per cent chance of not having the condition—and that this harm was definitely caused (on the balance of probabilities) by the negligence. Therefore, they had awarded him 25 per cent of the total damages he would have received had he been successful in his initial claim. However, this ‘lost chance’ idea was rejected by the House of Lords.⁵² They thought the question was whether Hotson was ‘doomed’ when he first came to the hospital (as in *Barnett*). It was most likely that he was, so the negligent treatment made no difference. Translating percentages into legal certainties, he had *no chance* of recovery.⁵³

Pause for reflection

Is it right that if a claimant can show a 51 per cent likelihood that negligence caused their harm it will be treated as a legal certainty that it did (and 100 per cent of the damages will be recoverable), whereas if you can show only a 49 per cent likelihood that the negligence caused the harm it will be treated as a legal certainty that it did not (and no damages will be recoverable)? This is known as the ‘all or nothing’ rule. It might seem fairer if we were talking about certainties, rather than probabilities.

It is unclear whether in *Hotson* the law lords meant to rule out permanently the possibility of ‘lost chance’ claims in medical/personal injury cases. Several cases post-*Hotson* recognised ‘lost chance’ when the claim was for economic loss,⁵⁴ so clearly the *type* of loss is recognised within the law of negligence (damages for lost chance are also recognised in contract law). In relation to medical negligence, the issue was revisited in *Gregg v Scott* [2005].

***Gregg v Scott* [2005] UKHL 2 (HL)**

The claimant consulted a doctor complaining about a lump under his arm, which the doctor failed to accurately diagnose as cancerous. As a result, the treatment the claimant received was delayed by nine months, and his chance of survival decreased.

Evidence showed that 17 in every 100 people with the same kind of cancer could be cured by prompt treatment (within one year of the tumour appearing) and a further 25 in every 100 people would be cured even with a delay of one year. The other 58 would be incurable, no matter when the cancer was diagnosed or treated. Using these figures, the claimant contended that he initially had a 42 per cent chance of being cured, had his treatment not been delayed, which decreased to 25 per cent because of the delay.

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↪ A 3:2 split House of Lords rejected his claim, the majority stating that the claimant needed to show that the doctor’s negligence made it more likely than not that he could not be cured. Since, at the outset, he was 58 per cent likely to die anyway, this was not possible.

In the alternative, the claimant argued that he had lost a 17 per cent *chance* of survival (the difference between 42 per cent and 25 per cent). This claim was also rejected, the majority holding that loss of chance could not form the basis of a medical negligence claim.

The ‘lost chance’ argument in *Gregg* differs from that in *Hotson* in an important way—in *Hotson* there was an either/or situation: when he reached hospital the first time it was either too late to do something about his hip, or it was not (see also *JD v Mather* [2012]). In *Gregg*, the situation was different: even when the claimant first went to the doctor the chances of survival were against him and, on balance, he was 58 per cent likely to die from the cancer. The delay in diagnosis had *reduced* his *chance of survival*, not eradicated it, raising the straightforward issue of whether a calculable lost chance is recoverable.⁵⁵

Counterpoint

Lord Nicholls, in the opening paragraph of his powerful dissent in *Gregg*, described the issue as follows:

This appeal raises a question which has divided courts and commentators throughout the common law world. The division derives essentially from different perceptions of what constitutes injustice in a common type of medical negligence case. Some believe a remedy is essential and that a principled ground for providing an appropriate remedy can be found. Others are not persuaded. I am in the former camp. (at [1])

Arguably, this passage sums up most of the law on factual causation—or at least those cases where an unorthodox approach has been taken or departure from the ‘but for’ test has been suggested. Lord Nicholls goes on to describe the idea that a patient can only recover damages if his original chance of recovery had been more than 50 per cent as not only ‘irrational and indefensible’ (at [3]) but makes the duty of care a doctor owes their patients to take reasonable care ‘hollow’ (at [4]). We would agree.

Baroness Hale (in the majority in *Gregg*), on lost chance, pointed out that if it could be concluded that a doctor’s negligence had affected a claimant’s chances to some calculable extent, then we would have ‘a “heads you lose everything, tails I win something” situation’, operating in favour of the claimant (at [224]). We suggest this is better than a ‘heads I win something, tails I win nothing’ situation, operating wholly in favour of defendants, particularly when they are clearly negligent.⁵⁶ The policy arguments supporting a claimant-favouring system stem from the underlying aims of the tort system itself, not least the desire to compensate those who have suffered harm due to the wrongs of another, but also in the more ‘practical’ aspects such as making people (in this example, doctors) act more *carefully* than they might otherwise, reducing the number of *future* potential claimants. Crudely, as the law stands, the ‘chance’ of the claimant making a financial gain (or avoiding an economic loss) is seemingly valued more highly than the chance of the claimant not suffering physical harm or even death.

Furthermore, the type of calculation that the court is asked to make in a loss of chance claim is not one that is wholly unknown to them (not only because lost chance is recognised in other types of claim, outside medical negligence or other personal injury). When damages for an ‘ordinary’ personal injury are calculated, reductions and increases in the amount to be awarded can be based on the ‘chance’ of something happening in the future.⁵⁷

More information on this is available in this chapter’s web links <https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-chapter-9-pointers-to-pause-for-reflection-and-counterpoint-boxes?options=showName>.

After *Gregg* it seems that claims for lost chance in personal injury/medical negligence are ruled out.⁵⁸

Pause for reflection

Do you think that the fact that Gregg was still alive at the time of the House of Lords decision counted against him? Had he actually lost the chance of survival (by dying), might the decision have gone the other way? Lord Phillips, who was in the majority, seemed to indicate that he might have ruled the other way (thus making the outcome 3:2 in favour of the claimant), had Gregg in fact died (at [188]).

p. 273 9.2.3 Multiple sufficient causes

We have already considered problems encountered where a claim involves multiple defendants who each *could have* caused the harm complained of, but cannot be proven to have done so, according to the ‘but for’ test. A similar but different problem arises where there is more than one defendant, each of whom *does* pass the ‘but for’ test, but where one of their actions comes later than that of the other. Or, the later action may be non-tortious, but still capable of giving rise to the harm. Here, instead of indeterminate or multiple *potential* causes, we have what are known as multiple *sufficient* causes; the question is whether the action of the second party (negligent or otherwise) absolves the original tortfeasor from blame. Put another way, should the original defendant, who is clearly liable for *some* harm, *remain* liable after the unrelated actions of another party?

***Baker v Willoughby* [1970] AC 467 (HL)**

The claimant’s leg was injured in a car accident negligently caused by the defendant. Resulting pain and stiffness meant that he had to find alternative employment. Later, while working in a scrap-metal yard, but before the case went to trial, he was shot in the same leg by armed robbers. His leg was later amputated. The question for the House of Lords was whether the negligent driver should pay compensation for the effect of his negligence (the pain and stiffness) for the rest of the claimant’s life, or whether his liability ended when the claimant was shot. The defendant argued that the shooting ‘obliterated’ the effect of the injury he had caused. This argument had been accepted by the Court of Appeal, which awarded damages for the leg injury only up until the time of the shooting. It was not in dispute that the robbers would have been liable for the additional extent of the injury they caused, but they were never found, so could not be sued.

The House of Lords felt that this would lead to a gap in damages. If the original defendant was liable for the original injury only to the point in time that a later injury caused by another party occurred and that party was only liable for the *additional* harm caused, then there would be no compensation for the continuing effects of the first injury. Therefore, they said that the original defendant’s negligence continued to operate even after the additional harm caused by the gunshot.⁵⁹

What is the position when the later event is either non-tortious or even naturally occurring? In such a situation, no one would be liable for the additional harm caused by the later event.

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***Jobling v Associated Dairies Ltd* [1982] AC 794 (HL)**

The claimant injured his back at work, due to the negligence of his employer. Before trial, he contracted an unrelated back disease which rendered him completely incapable of work. As in *Baker*, the question for the House of Lords was whether the defendant should remain liable for the original injury and the loss of earnings associated with it past the point where the later (and greater) harm was suffered.

The House of Lords refused this claim, relying on the fact that when assessing damages account can be taken of the ordinary ‘vicissitudes of life’ (hardships) that a person might be expected to encounter, and that damages are usually reduced accordingly to take account of these. This meant that *Jobling* could only be compensated up until the point that he contracted the disease, which would have happened anyway.

Jobling is a case where the later ‘sufficient cause’ of the harm complained of overtakes or *supersedes* the original cause. When this happens, the defendant will be liable only for the damages relating to the extent of the injury they caused, up until the time of the superseding event. The point is that the superseding event, such as the disease in *Jobling*, would have happened anyway, whether the claimant was previously injured or not.

While *Jobling* did not explicitly overrule *Baker*, it is probable that *Baker* is now ‘confined to its facts’. It is therefore possible that if a similar situation arose today, the later event, even if tortious or criminal, would supersede the first.⁶⁰ The question whether or not *Baker* ‘survives’ *Jobling* may in part be answered by *Murrell v Healy* [2001]. The claimant was injured in a car accident, for which he received £58,500 compensation from the other driver in an out-of-court settlement. Later the same year, he was involved in another accident—the court was asked whether the defendant should pay a reduced sum of damages to make allowance for the fact that the claimant had already been compensated for *some* injury. The Court of Appeal held that the defendant should compensate Murrell only for additional damage caused by the accident (the reverse of *Baker*, where the second tortfeasors were unable to be sued) and that if the first defendant’s liability was unaffected by the second tort, the second defendant cannot be liable for any of the original losses. Murrell also developed joint problems in his legs which were unattributable to either of the accidents—following *Jobling* and the ‘vicissitudes of life’ idea, the damages he had been awarded were then reduced.⁶¹

Pause for reflection

Note that the question whether something is a supervening cause is very similar to the question whether the actions of a later tortfeasor (or otherwise) break the chain of causation back to the original defendant (thereby absolving them entirely from liability). Where the line is drawn between these two points is very hard to define, and people have different opinions—where do you think it is? Is *Murrell* a case where the chain of causation would have broken, but the claimant was lucky enough to have already settled out of court with the first defendant?

Test your understanding by trying this section's self-test questions <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-self-test-questions-9-2>.

p. 275 9.3 Legal causation

Once factual causation has been established, the 'real' or 'operative' cause in law (legal causation) must still be determined. This involves two distinct tests, the first asking whether the harm was too 'remote' a consequence of the defendant's negligence, and the second asking whether any subsequent event could be said to have broken the chain of causation stemming from the defendant's breach. Each test represents a value judgement about how far we think defendants' liabilities should extend—that is, regarding the final question about to whom responsibility for harm is appropriately allocated.

9.3.1 Remoteness of damage

When a court asks whether a harm was too remote a consequence of the defendant's negligence, it is essentially asking whether the consequences of the breach were so far removed from it as to have been *unforeseeable* by the defendant (judged by the standard of the 'reasonable person') at the time. Put another way, the defendant may argue that the consequences of their action were *not* foreseeable—that is, that they were too remote.⁶² If so, cause *in law* is not established and the claimant's case fails. Whether something is too remote is judged considering what was known at the time the breach occurred; that is, with foresight, not hindsight.

Overseas Tankship (UK) Ltd v Morts Dock and Engineering Co (The Wagon Mound) (No 1) [1961] AC 388 (PC)

The defendants were responsible for a ship that had been loading in Sydney Harbour, Australia. Due to the negligence of a crew member, oil leaked into the water and formed a thin film over its surface, spreading to a neighbouring wharf owned by the claimants, where welders were working repairing another ship. Some days later, sparks from the welding ignited the oil—the resulting fire caused serious damage to the wharf and other nearby ships.

At first instance, the damage caused by the oil itself (e.g. to the slipway of the wharf) was deemed reasonably foreseeable but, because evidence showed that oil spread thinly on water is difficult to ignite, damage to the claimant's property caused by fire would *not* have been foreseeable to a reasonable person (at the time the oil was spilt). The Privy Council agreed, finding that a reasonable person could not have foreseen fire damage as a possible consequence of the negligent act.⁶³

p. 276 ↩ Before *The Wagon Mound (No 1)*, a defendant was liable for *all* direct physical consequences of their negligent act (*Re Polemis and Furness, Withy & Co Ltd* [1921]). This meant that defendants would be liable for any direct consequences of their breach, whether or not these could have been foreseen, even if the consequences may have been substantially different from those a reasonable person might expect, and even if they ended up being far more serious than could have been anticipated. *The Wagon Mound (No 1)* substantially narrowed the *Re Polemis* test (indeed, the Privy Council stated that *Re Polemis* was no longer good law), establishing a far more defendant-friendly test. Following *The Wagon Mound (No 1)*, the question asked to establish whether the claimant's harm is too remote is: 'Was the *kind* of damage suffered by the claimant reasonably foreseeable at the time the breach occurred?' This remains the primary test for remoteness of damage in negligence—and indeed extends further into other torts.⁶⁴ *The Wagon Mound (No 2)* [1967] did not change the nature of this test—it is simply that on the new evidence presented to the court by this time, the Privy Council could conclude that even a thin film of oil on water would ignite—although the risk of damage by fire was small, it still existed.

The way the remoteness test is formulated means that only the *type* of damage must be foreseeable, not the *extent*. So, if *some* physical damage is a foreseeable consequence of the breach, the *amount* of physical damage suffered need not be.⁶⁵ Take a simple example of driving a car. It is clear to any reasonable person that if a car is driven negligently, physical injury to other road users (including other drivers, passengers, cyclists, pedestrians and so on) is foreseeable, as would be property damage to any other vehicle. So, it does not matter in law that, even with a small amount of carelessness (e.g. a momentary lapse of concentration), a pedestrian is seriously injured or even killed. The equation is:

NEGLIGENT DRIVING=FORESEEABLE RISK OF PHYSICAL INJURY=LIABLE FOR ALL PH

Similarly, if a negligent driver hits another person's vehicle, it is evident that damage to the vehicle itself would be foreseeable, as would be damage to the vehicle's contents. It would not matter what the contents of the vehicle are—so if a truck was transporting cheap beans or expensive designer clothing, the negligent driver would be liable for the full cost of whatever was damaged.

While this distinction (type versus extent of damage) appears relatively straightforward, it has not always proved to be so. In *Doughty v Turner Manufacturing Co* [1964], a factory worker suffered serious burns when an asbestos lid was knocked into a vat of hot liquid, causing a chemical reaction that made the liquid boil up over the edge. The specific chemical reaction was not foreseeable to the reasonable person—but the claimant contended that as it would be foreseeable that the lid falling in would cause some liquid to splash out, and that the likely result of this would be that the claimant would be burnt, the *type of injury* he had suffered was foreseeable. However, the court disagreed with this interpretation, finding that an eruption of boiling liquid was different from being splashed by liquid.

p. 277 Another case, however, appears to tell a different story. In *Hughes v Lord Advocate* [1963], an 8-year-old boy fell down a manhole in the street which had been left open by Post Office employees earlier that day. Upon finishing work, the men had covered the hole with a canvas tent and surrounded this with paraffin lamps to alert to the danger. The boy picked up a lamp and took it into the tent. While playing, he knocked the lamp into the manhole, which caused an explosion of paraffin vapour, and the boy fell into the hole and was badly burnt. The defendants claimed that they could not have foreseen that the boy would enter the tent with the lamp and be burnt by a subsequent explosion. The House of Lords disagreed. Finding the defendants liable, they held that it was reasonably foreseeable that the *type* of damage would be burns, therefore the exact means by which the burns occurred did not matter (see also *Jolley v Sutton London Borough Council* [2000]; *ST v Maidstone and Tunbridge Wells NHS Trust* [2015]; *Hicks v Young* [2015]).

Pause for reflection

How can the difference between the two previous cases be explained? Do you think the fact that both cases were decided in the two years immediately after *The Wagon Mound (No 1)*, which created a 'new' test, could have anything to do with it? Perhaps the judges were struggling to implement the new test. Or perhaps the court was simply more sympathetic towards a child?

In the years following the reformulation of the remoteness test in *The Wagon Mound (No 1)*, there were numerous inconsistencies in attempts to convince a court that physical damage suffered was too remote to be attributed to the defendant. For example, in *Tremain v Pike* [1969], a defendant negligently allowed his farm to become infested with rats. The claimant farm worker contracted Weil's disease because of exposure to rats' urine. Although clearly arguable that physical injury from rats is foreseeable, the claimant lost because the court held that while injury from rat bites would be foreseeable, Weil's disease was not. However, in *Vacwell Engineering Co Ltd v BDH Chemicals Ltd* [1971], it was held that because a small chemical explosion might be foreseeable, all damage caused by a large explosion that occurred when ampoules of chemicals were being washed in water was foreseeable even though no one could have predicted this happening.

Pause for reflection

It seems unlikely that a court today would follow the ‘version’ of the foreseeability test seen in *Doughty*. Mark Lunney, Donal Nolan and Ken Oliphant suggest that the ‘modern trend’, at least in relation to personal injuries, seems to be to interpret the *type* of injury in a broad sense (see e.g. *Smith v Leech Brain* [1962], discussed later), so the extent of the harm or the exact way in which it was caused poses no problems to claimants.⁶⁶ This may also explain the decision to include psychiatric harm with physical injury under the more generous heading of ‘personal injury’ in *Page v Smith* [1996] and *Simmons v British Steel plc* [2004].⁶⁷

p. 278 9.3.1.1 The ‘egg shell skull’ rule

We have seen that the remoteness test dictates that once the type of harm is judged foreseeable, the defendant incurs liability for all damage caused, even where its extent was not foreseeable. A further rule has been developed to protect even those claimants who suffer what may be called ‘extreme’ damage because they already have some kind of susceptibility or ‘weakness’.

The ‘egg shell skull’ rule, as it has become known, is simply a principle that states that a defendant will be liable for the full extent of the harm they caused, even if the person harmed suffered more harm than might be expected, due to existing weaknesses or frailty. The maxim is expressed as ‘the defendant must take the victim as they find them’. The classic example comes from *Smith v Leech Brain & Co Ltd* [1962] where, because of the defendant’s negligence, the claimant was burnt on the lip. The claimant had a pre-cancerous skin condition, which became cancerous because of the burn, and he eventually died. The defendant was held liable for the full consequences of his negligence (i.e. all the physical damage (*type*), so this included the cancer (*extent*)). The court held that because physical injury had been foreseeable, it did not matter what the extent of this was, or that the claimant was already more susceptible to a greater level of harm. This approach was followed in *Reaney v University Hospital of North Staffordshire NHS Trust* [2014], but on appeal the Court of Appeal described the correct approach as objectively determining whether the claimant’s needs before and after the negligently caused injury were ‘quantitatively’ or ‘qualitatively’ different (see [19]). If quantitatively different (i.e. more of the same), then a defendant is only liable for any additional needs flowing from their negligent act. If qualitatively different (i.e. different in kind), then a claimant may recover in full.⁶⁸

In *Lagden v O’Connor* [2003] it was confirmed that the ‘egg shell skull’ principle also applies to economic harms. Thus, if a claimant suffers heightened economic loss because of already weak economic circumstances, a defendant will be liable for the full extent of that loss.

9.3.2 Intervening acts

Even if the defendant ‘causes’ an accident by starting a sequence or chain of events, a later event might be held to be the ‘real’ cause of the eventuating harm. In such circumstances, the ‘chain of causation’ (which began with the defendant’s breach) is said to be ‘broken’, leaving the defendant not liable for the claimant’s harm. The later event is known as a *novus actus interveniens* (literally, a new intervening act, already familiar to those who have studied criminal law).

This is the second aspect to establishing cause in law (the first being to assess whether the harm was not too remote). However, there is a very fine line between ‘supervening’ events (as seen in *Jobling* and which go to questions of cause in *fact*) and later ‘intervening’ acts (which involve questions relating to cause in *law*).

9.3.2.1 Later negligent acts

One question is what *kind* of later act is required for the chain of causation to be broken. It is clear that subsequent *deliberately* wrongful acts (e.g. crimes) will be sufficient (*Weld-Blundell v Stephens* [1920]).⁶⁹

p. 279 Beyond this, the question remains whether a later act must be *negligent* ↵ in order to break the chain of causation. Whether what happens subsequently is foreseeable is also relevant—if it is something that is clearly likely to happen because of the defendant’s negligence then it is unlikely to break the chain. This was plainly expressed by Lord Reid in *Home Office v Dorset Yacht Co Ltd* [1970], who said that a subsequent negligent act of a third party must ‘have been something very likely to happen if it is not to be regarded as a *novus actus interveniens* breaking the chain of causation. I do not think a mere foreseeable possibility is or should be sufficient’ (at 1030).⁷⁰

In *Webb v Barclays Bank and Portsmouth Hospitals NHS Trust* [2001] it was established that later medical negligence will not usually break a chain of causation back to an original defendant’s breach unless it is ‘so grossly negligent as to be a completely inappropriate response to the injury by the defendant’. The Court of Appeal in *Webb* held that the original wrongdoing remained a causative force. An intervening act must be something very unforeseeable (*Dorset Yacht*) and the usual position (sadly) is that ‘ordinary’ medical negligence is not unforeseeable. However, this position was challenged in *Jenkinson v Hertfordshire County Council* [2023], where Baker J in the High Court said that medical negligence did *not* need to be gross to break the chain of causation (overruling a District Judge who had relied on *Webb*). He found that there was no logical justification for creating a specific rule for medical negligence—instead the question was whether the ‘claimant’s initial injury, admittedly the result of the defendant’s negligence, was so badly mistreated that the defendant ought not, in fairness, to be considered responsible for the consequences of that mistreatment’ (at [44]). Whether this displaces the principle from *Webb* remains to be seen.

In *Rouse v Squires* [1973] a lorry driver negligently caused his lorry to jack-knife and block part of a motorway. A car subsequently drove into the rear of the lorry. A second lorry driver stopped to assist, pulling over some distance in front of the accident. A third lorry driver stopped before the accident scene, leaving his headlights on for illumination. The driver of a fourth lorry was less careful and did not see the accident until he was almost upon it. He skidded into the third lorry, killing its driver. The question for the court was which negligent lorry driver caused the death. At first instance, the trial judge held the fourth lorry driver wholly

responsible as his negligent driving had broken the chain of causation in relation to the original negligent lorry driver. However, the Court of Appeal found that the chain of causation had not been broken. *Both* drivers were therefore responsible, and damages were apportioned between them.

***Knightley v Johns* [1982] 1 WLR 349 (CA)**

A serious traffic accident in a one-way tunnel was caused by the defendant's negligent driving and police were called to the scene. Having forgotten to stop traffic entering the tunnel, the police inspector in charge sent two officers on motorcycles, one of whom was the claimant, against the direction of the oncoming traffic to go back and close the tunnel. Both the instruction and driving against the flow of traffic were expressly contrary to police standing orders, but nevertheless the two officers complied with the instruction. Near the entrance to the tunnel, the claimant suffered injury when he was hit by a car.

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↵ At first instance, the judge found that neither the inspector's instruction nor the claimant had been negligent and that their actions did not break the chain of causation back to the original defendant (the driver), who would therefore be liable for the claimant's injuries in full. However, the Court of Appeal found that the inspector's instruction *had* been negligent and that this was sufficient to break the chain of causation between the defendant car driver's original negligence and the claimant's injuries. As a result, no claim could succeed against the defendant.

Where more than one party can be said to be responsible for causing harm, it is unusual for the courts to find only one of them legally responsible. The Civil Liability (Contribution) Act 1978 means that the court can apportion damages between the parties responsible (or leave them to do this for themselves). Because of this, it is not common for a court to find that a later negligent act breaks the chain of causation back to the original defendant.⁷¹

In *Wright v Lodge* [1993],⁷² the first defendant's Mini broke down on a dual carriageway and she negligently failed to move it to the hard shoulder. While she was trying to restart the car, it was hit by a lorry being driven negligently over the speed limit, injuring a passenger in the car as a result. The lorry skidded across the central reservation, tipped on its side and collided with oncoming vehicles on the other side of the carriageway, injuring two drivers, one fatally. The injured parties sued the driver of the lorry, who joined (using the Civil Liability (Contribution) Act 1978) the driver of the Mini as a co-defendant. For her negligence, she was ordered to pay 10 per cent of her passenger's damages (the lorry driver paid the remainder). However, with respect to the injuries to the other claimants, the lorry driver was found to have broken the chain of causation from her negligence and, as a result, was wholly liable.⁷³

9.3.2.2 Acts of the claimant

Can the claimant's own negligence break the chain of causation? It is rare for this to be the case, because generally if a claimant negligently contributes to their own injuries, this can be dealt with via the defence of contributory negligence.⁷⁴ Since 1945 this has been only a 'partial' defence, meaning that the claimant's contribution to their own harm can be assessed and their damages reduced accordingly.⁷⁵ We know that 'mere unreasonable conduct on a claimant's part' is unlikely to break the chain.⁷⁶ However, there may still be occasions where the claimant's own actions after the defendant's original negligence are considered so careless that they should remove all responsibility from the defendant. In *McKew v Holland and Hannen and Cubitts (Scotland) Ltd* [1969], the claimant suffered a leg injury at work due to his employer's negligence, which resulted in stiffness and impaired mobility. Subsequently, p. 281 he went to inspect a flat in the course of his work, which could only be accessed by descending a steep staircase without a handrail. As he was about to descend the stairs, his leg buckled and to avoid falling down the stairs and landing on his head, he threw himself forwards, breaking his ankle. McKew claimed that the defendant should be liable for this damage, as it was an extension of the original injury. However, the House of Lords rejected his claim. The claimant had negligently thrown himself down the stairs and had broken the chain of causation. The defendant was not, therefore, liable for his broken ankle.

McKew can be contrasted with *Wieland v Cyril Lord Carpets* [1969] where, because of the defendant's negligence, the claimant had to wear a neck brace. This restricted her ability to use her bifocal glasses properly with the result that she fell down some stairs, injuring her ankle. The court found that she had been as careful as she could have been and therefore her actions could not break the chain of causation and her ankle injury was also caused by the defendant's negligence.

Pause for reflection

Do you think the difference between the two cases just described can be attributed to the amount of care the claimants took for themselves after they had sustained their original injury? Was it not reasonable for McKew to choose to fall in the way he did rather than risk suffering a different type of injury? What do you think the situation would have been if McKew's injured leg had buckled completely while on his way down the stairs? See *Spencer v Wincanton Holdings Ltd* [2009], where the claimant, who had failed to secure his prosthetic leg when he stopped to refuel his car, fell over, exacerbating an existing serious injury. The Court of Appeal found that Spencer had not acted so unreasonably that he had broken the chain of causation; contributory negligence was appropriate to deal with the sharing of responsibility.⁷⁷

A claimant's own actions in committing suicide while imprisoned (*Kirkham v Chief Constable of Greater Manchester Police* [1990]) or while being held in police custody (*Reeves v Commissioner of Police for the Metropolis* [2000]) have been found not to break the chain of causation back to the defendant. While this might seem surprising, as suicide is a deliberate act, the reasoning of the courts was that because the police had a duty of care to protect the specific claimant from suicide (in *Kirkham* by passing on the information

that the prisoner was depressed and may become a suicide risk), the claimant doing exactly what may be expected of them should not negate that duty. In *Reeves*, however, the damages awarded were reduced for the contributory negligence of the claimant.

p. 282 In *Corr v IBC Vehicles Ltd* [2008], a similar question arose. A man was seriously injured and almost killed in an accident at work. The accident and injuries led to him suffering clinical depression, and he later committed suicide. His employers (the defendants) did not, as in *Reeves* and *Kirkham*, owe him a specific duty of care to prevent suicide. Instead, the question was whether the suicide had been caused by the original breach of duty (to keep employees reasonably safe) or whether the claimant's deliberate action had broken the chain of causation. The House of Lords found that the chain of causation was not broken; Lord Bingham in particular was swayed by the idea that the suicide was not a free and voluntary choice *because* of the effect of the accident on the mind of the claimant (at [16]). In contrast, the House of Lords found in *Gray v Thames Trains* [2009], discussed in Chapter 10, that a later *criminal* act committed by the claimant, who had sustained both physical and psychiatric injuries in a train crash negligently caused by the defendants, did break the chain.

Test your understanding by trying this section's self-test questions <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-self-test-questions-9-3>.

9.4 Conclusion

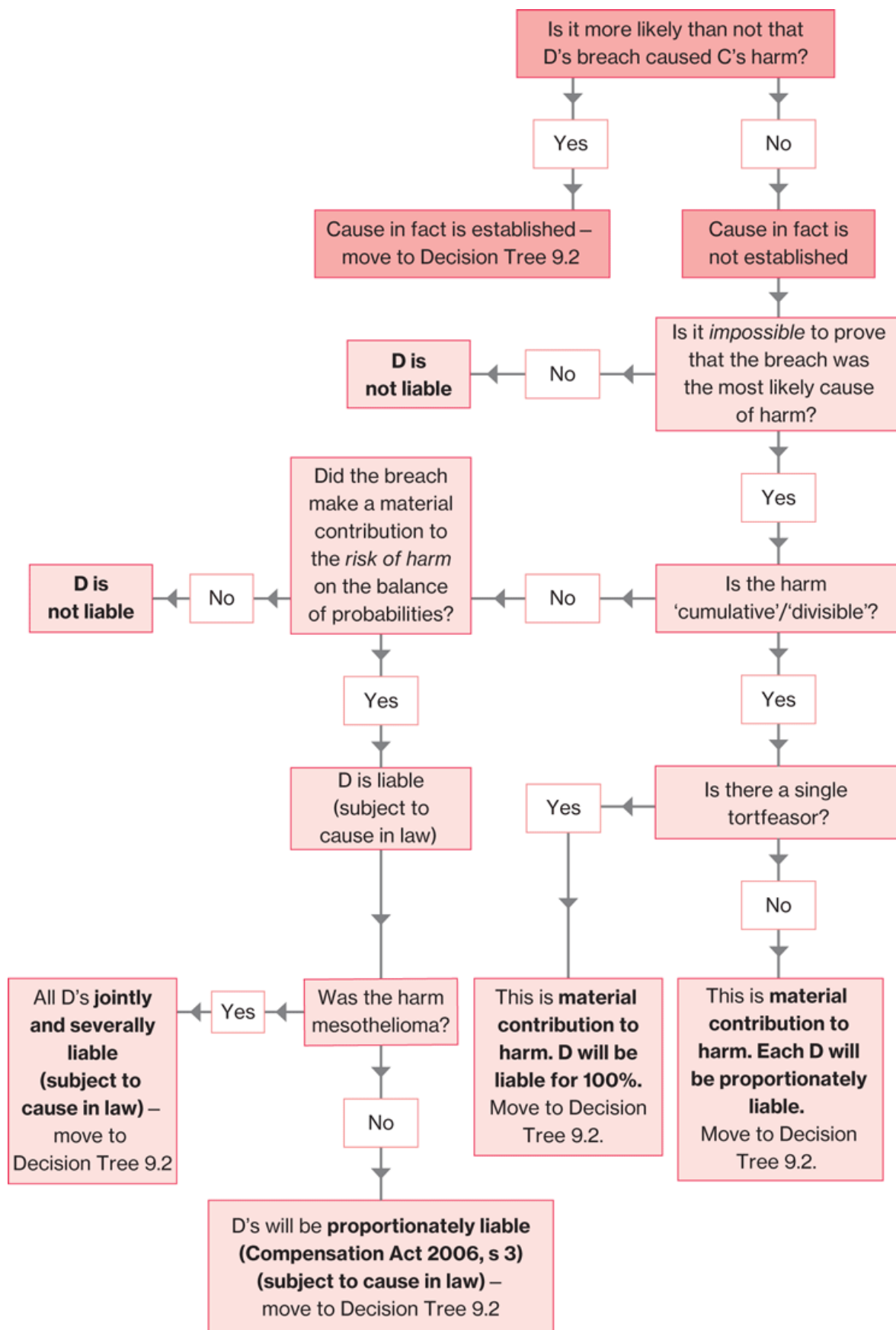
After a claimant establishes breach of a duty of care owed to them by the defendant, the next 'hurdle' is establishing *causation*. This has two aspects: *cause 'in fact'* and *cause 'in law'*.

To establish cause in fact, the claimant must show, on the balance of probabilities, that the defendant's *breach* caused their harm. This is often expressed as a 'but for' test: can the claimant establish that 'but for' the defendant's negligent act, they would not have suffered harm? There are many flaws with this test. Its inflexibility can cause unjust outcomes, especially when there are multiple potential causes of the harm. The courts have therefore devised exceptions to the test, used in specific types of exceptional case, usually where the outcome that would be generated by the 'but for' test is deemed to be unsatisfactory in some way. However, it should be remembered that the principal exceptions to the 'but for' test detailed here do not and cannot apply in all cases. To a very large extent they are limited to the type of case in which the decision was first made (e.g. the material contribution test originated from industrial cases, and is only now beginning to apply in some medical negligence cases; and 'lost chance' claims seem limited to claims for economic loss, having been ruled out in medical claims).

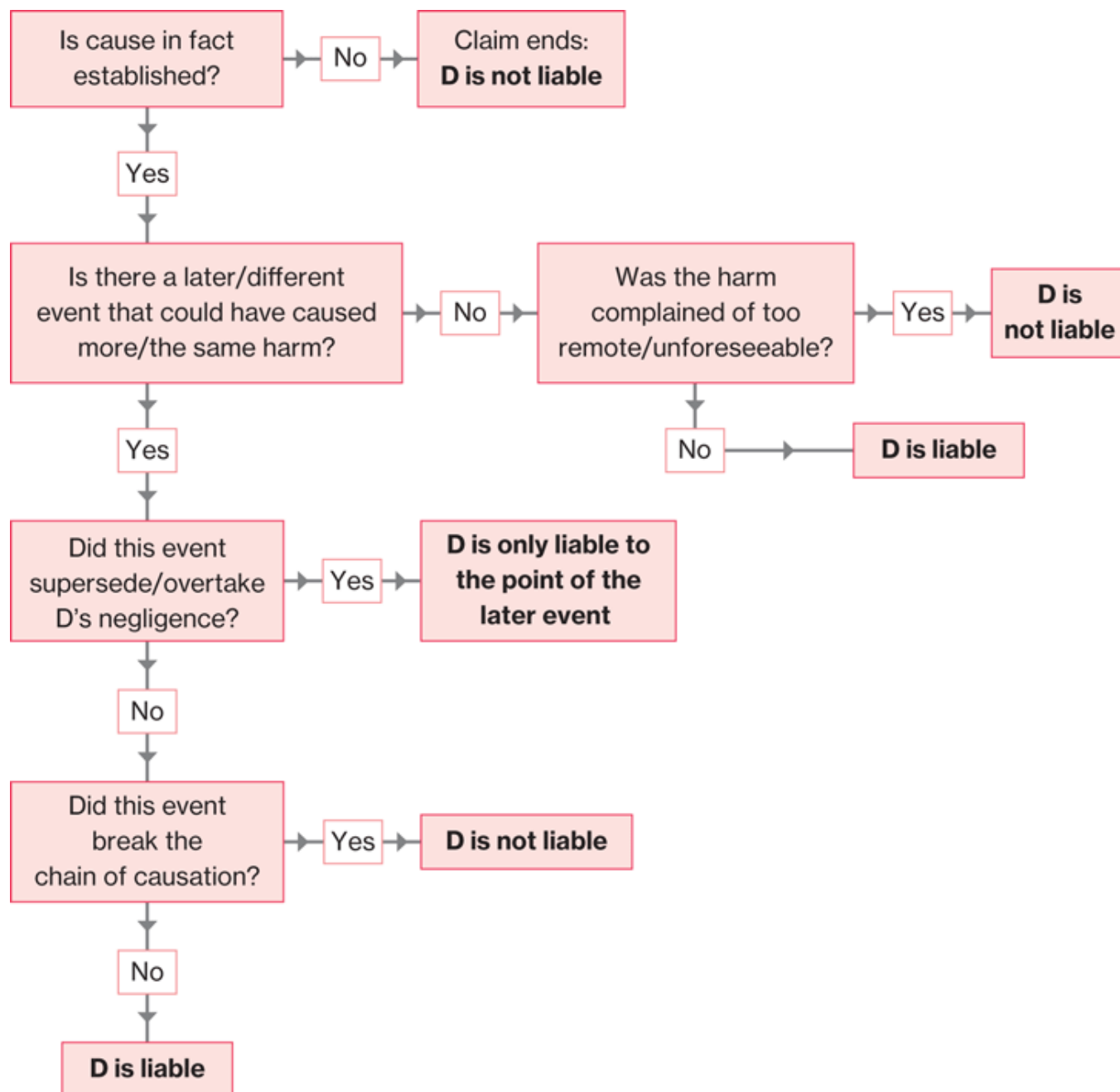
The second aspect of causation is cause in law, which has two parts (though it may be that only one of them is relevant/arguable in a particular scenario). They encompass a *remoteness* test (which involves establishing whether the damage that occurred was foreseeable to the defendant at the time of the negligence) where it is the *type* of harm (e.g. personal injury) that must be foreseeable, not its *extent*. The second test is to ask whether any *intervening acts* (acts that occurred after the defendant's breach) broke the chain of causation. If so, the defendant will not be liable.

Decision Trees 9.1 and **9.2** show how you work through cause in fact and cause in law questions.

Explore the elements of and difficult pathways through causation using the interactive versions of **Decision Trees 9.1** <<https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-chapter-9-interactive-decision-tree-9-1>> and **9.2** <<https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-chapter-9-interactive-decision-tree-9-2>>.



Decision Tree 9.1 Establishing cause in fact



Decision Tree 9.2 Establishing cause in law

p. 283 End-of-chapter questions

After reading the chapter carefully, try answering the questions which follow.

- 1 Is there a coherent way of explaining the difference between supervening and intervening acts?
- 2 Does the remoteness test lose some of its impact or validity with the incorporation of the 'egg shell skull' rule?
- 3 What policy reasons can you identify that have affected the courts' decisions in asbestos-related injury cases? Are they consistent?

- 4 To what extent do the rules on factual and legal causation help to achieve the general aims of tort law?

If you would like to know what we think, check the **answer guidance** <https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-chapter-9-answers-to-end-of-chapter-questions?options=showName>.

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Answering the problem question

Consider again the problem question at the start of this chapter. Now having read about the topic, **what would be your advice to Gavin?**

Here are some pointers to get you started:

- It seems obvious that S will owe G a duty of care and has breached this. The question is whether G's broken arm is a 'but for' cause (cause in fact) of S's negligence.
- G will also sue C. If a breach could be established, this claim raises causation points similar to those seen in **Bolitho**. There is also a 'but for' issue raised by the evidence of the expert witness.
- If the 'but for' test can be satisfied in the previous claims, the issue of foreseeability of harm will arise (cause in law), as will potential intervening acts. Did, for example, C's treatment of G break the chain of causation back to S?
- Another question is how the later injury caused by the speeding motorist affects any claim. Is this a supervening event which obliterates any earlier causes (see **Jobling**)?

If you need some more guidance:

- See the **annotated version of the problem with issues and cases to consider** <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-chapter-9-annotated-problem-question-interactive>.
- Check your answer against the **suggested outline answer** <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-chapter-9-answers-to-problem-question>.

Further reading

Reading on causation is often quite difficult, and many articles focus solely on one or two cases. Reading these—and case notes—is good to gain a detailed understanding of some of the more difficult or controversial cases. Many of the articles and case notes listed below focus on a particular case or cases.

Bailey, Stephen 'Causation in Negligence: What is a Material Contribution?' (2010) 30 Legal Studies 167

Hoffmann, Lord '*Fairchild* and After' in Andrew Burrows, David Johnston and Reinhard Zimmermann (eds) *Judge and Jurist: Essays in Memory of Lord Rodger of Earlsferry* (OUP 2013)

Laleng, Per 'Causal Responsibility for Uncertainty and Risk in Toxic Torts' (2010) 18 Tort Law Review 102

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Additional suggestions for further reading can be found in this chapter's **web links** <<https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-chapter-9-web-links?options=showName>>.

Notes

¹ Note, however, the recent decisions (handed down jointly) of the Supreme Court in *Manchester Building Society v Grant Thornton* [2021] and *Khan v Meadows* [2021] which sought to set out clearly the 'steps' to be taken in analysing any claim in negligence (the specific decisions are more relevant to the concept of 'scope of duty', considered in **section 2.5.4**). There is some difference in the way the tests for causation (i.e. the questions that must be asked) are

formulated in the separate judgments. We prefer the approach of Lord Burrows (at [79]–[80]) to that in the main judgment (at [28]), though it must be noted that, unlike us, he treats ‘remoteness’ as a separate question from ‘legal causation’ (our questions 2 and 3 in **Table 9.1**).

² HLA Hart and AH Honoré *Causation in the Law* (2nd edn OUP 1985).

³ The same rule applies in every tort where proof of harm or damage is required, so what is said in this chapter about causation as it relates to negligence should also be remembered in the context of other fault-based torts, as well as some strict liability ones (e.g. claims under the Consumer Protection Act 1987).

⁴ Questions of cause in relation to a defendant’s *failure* to do something only arise if there was a duty to act—see **Chapter 4**.

⁶ See also *Cunningham v Rochdale Metropolitan Borough Council* [2021], where the Court of Appeal found that despite a breach of duty of care owed by a school to one of its employees, the claimant could not establish that the harm (an attack by a pupil) would not have happened anyway.

⁵ Thus we would arrive at the same outcome in respect of Peter and his food poisoning in the chapter-opening scenario. Also, although the facts seem different on first glance, the same would apply to Sriya (see **Hotson**).

⁷ See e.g. *Schembri v Marshall* [2020], in particular at [51]–[56].

⁸ Though see the Court of Appeal decision in *Bailey v Ministry of Defence* [2008]. Here, there were two potential causes of the claimant’s cardiac arrest and brain damage—one negligent and one ‘natural’—and it could not be established which was the ‘but for’ cause. Following **Wilsher**, there should have been no liability. However, the Court of Appeal, treating the case as one of *cumulative* (indivisible) cause (see later), found that the hospital’s negligence made a ‘material contribution’ to the harm. See also *Leigh v London Ambulance Service* [2014], *Kennedy v London Ambulance Service NHS Trust* [2016] and *Leach v North East Ambulance Service NHS Foundation Trust* [2020] EWHC 2914 (QB).

⁹ In fact, the medical evidence said that it made a *difference*; but because the case was argued as ‘all or nothing’, what it could not say was whether the entire disease would have been avoided in the absence of the ‘guilty’ dust. This case makes much more sense if the language of ‘divisible vs indivisible injury’ is used (which was not the language used in the 1950s).

¹⁰ *Bailey v Ministry of Defence* [2008] (at [46]). Confirmed in *Garner v Salford County Council* [2013] where the claim failed for lack of evidence that the defendant’s negligence exposed the claimant to more than minimal levels of asbestos. The opposite conclusion was reached, in relation to a small proportion of total exposure, in *Carder v The University of Exeter* [2016].

¹¹ See also **section 9.2.2.2**.

¹² A good recent definition was given by Stuart-Smith LJ in **Holmes v Poeton Holdings Ltd** [2023] at [31].

¹³ See also *Ellis v Environment Agency* [2008]. Cf *Simmons v British Steel* [2004] in which the *Bonnington* test was used to hold the defendant fully liable for the (psychiatric) injury suffered by the claimant.

¹⁴ See also *Allen v British Rail Engineering* [2001], a case relating to ‘vibration white finger’, *Wright (a child) v Cambridge Medical Group* [2011] (baby with severe chicken pox symptoms negligently not referred to hospital by doctor, then negligently treated when eventually admitted to hospital, resulting in permanent hip damage); *Rahman v*

Arearose Ltd [2001] (claimant beaten then later receiving negligent medical treatment, both cumulatively found to have caused him psychiatric harm); and *Hatton v Sutherland* [2002] (a stress-at-work case) (*obiter*). This approach to apportionment in relation to *psychiatric* harm was later doubted: see *Dickins v O2 plc* [2008].

¹⁵ This issue has also been recently considered by the Court of Appeal in the context of a medical negligence claim. Handed down just over one week before *Holmes*, *CDE v Surrey and Sussex Healthcare NHS Trust* [2023] allowed an appeal and directed a rehearing based on the question whether a revised finding of fact that a consultant would have attended a difficult birth one minute earlier could make a material difference to causation in that case. Similarly, in *CNZ v Royal Bath Hospitals NHS Foundation Trust* [2023], an appeal was allowed by Ritchie J ‘due to the significant public interest and effect on the public purse potentially caused by his findings on material contribution’—an appeal decision is awaited.

¹⁶ Also it should be remembered that, following *Holtby*, defendants will only be liable for the extent of their contribution to a *divisible* injury. Where the harm is indivisible, if material contribution can be shown, the defendant will be liable for the full extent.

¹⁷ See e.g. Lord Bridge at 1090.

¹⁸ Also see Swift J in *ST v Maidstone and Tunbridge Wells NHS Trust* [2015] at [195]–[198].

¹⁹ Note, however, that the ‘single fibre’ theory has since been discredited by experts (*Sienkiewicz v Grief (UK) Ltd* [2011]; *Durham v BAI (EL Trigger Litigation)* [2008], later *BAI (Run Off) Ltd and others v Durham and others* [2012]). It is enough that the harm is ‘indivisible’ and that the state of scientific knowledge renders proof of causation impossible.

²⁰ Though now cf *Willmore v Knowsley Metropolitan Borough Council* [2011].

²¹ This may mean that Bern, in the scenario at the start of this chapter, would have a successful claim.

²² Perhaps it is the small difference in the facts—that there were only two potential causes, rather than five—that led to the Court of Appeal’s finding (distinguishing *Wilsher*) that there *could* be liability based on ‘material contribution’ in *Bailey v Ministry of Defence* [2008]. Though cf *Jones and others v Secretary of State for Energy and Climate Change and another* [2012].

²³ *Ministry of Defence v AB and others* [2010] at [154].

²⁴ *Jones and others v Secretary of State for Energy and Climate Change and another* [2012] (at [6.18]). As WE Peel and James Goudkamp point out, however, ‘one obstacle is the decision in *McGhee*, which was not a mesothelioma case’ (*Winfield & Jolowicz on Tort* (19th edn Sweet & Maxwell 2014) [7-021]).

²⁵ The point is that although mesothelioma is indivisible, if the harm is construed as ‘risk of damage’, that is divisible.

²⁶ A principle confirmed in *Equitas Insurance Ltd v Municipal Mutual Insurance Ltd* [2019].

²⁷ See also Peel and Goudkamp **n24** [7-022], who say ‘given the uncertainty surrounding the *Fairchild* exception and the quixotic process it unleashed, it may be that the law should have been left unaltered while the state of scientific knowledge was left to develop, notwithstanding the policy based impulse for alteration’. A similar view was expressed by the Court of Appeal in *Equitas Insurance Ltd v Municipal Mutual Insurance Ltd* [2019]. Males LJ said that ‘once the courts can be confident that the objective of ensuring victim protection has been achieved, it is desirable that the anomalies should be corrected and that the law should return to the fundamental principles of the common law. Put shortly, once unorthodoxy has served its purpose, we should revert to orthodoxy’ (at [91]).

²⁸ Or, more importantly perhaps, to claimant lawyers and unions.

- ²⁹ Section 3(1)(a)–(d). Thus, in the scenarios at the beginning of this chapter, Bern’s claim will not be unduly affected by **Barker**.
- ³⁰ See **Chapter 21**.
- ³¹ Though they may have recourse to some damages via the Financial Services Compensation Scheme.
- ³² Compare two decisions of the High Court of Australia, however: *Amaca Pty Ltd v Ellis* [2010] (where the claimant was also a smoker), distinguished in *Amaca Pty Ltd v Booth* [2011].
- ³³ Though see the comments (*obiter*) of Smith LJ in *Novartis Grimsby Ltd v Cookson* [2007], a bladder cancer case, in which she indicates that she would have applied the **Fairchild** principle in the absence of an (almost straightforward) application of the ‘but for’ test, as the two potentially causative agents ‘act on the body in the same way’ (at [72]).
- ³⁴ See e.g. *Clough v First Choice Holidays* [2006]; *Sanderson v Hull* [2008]; *Wootton v J Docter Ltd* [2008]. Note that it has been confirmed that the **Fairchild** test does not change how the duty is formulated and, to get to the stage of applying it, the duty and breach elements of a claim must be established in the ordinary way: *Williams v University of Birmingham* [2011].
- ³⁵ In *Employers’ Liability Insurance ‘Trigger’ Litigation: BAI (Run Off) Ltd and others v Durham and others* [2012] (‘Trigger Litigation’). In *International Energy Group Ltd v Zurich Insurance plc* [2013], the claimants contended that **Barker** should continue to be applied to a claim arising in Guernsey (where s 3 of the Compensation Act 2006 does not apply). The Court of Appeal ruled that the approach of the Supreme Court in *BAI (Run Off)* (based on Lord Rodger’s approach in **Barker**) was to be preferred. Following an appeal heard in July 2014, with further submissions in January 2015, a seven-member Supreme Court confirmed that this is the position (*Zurich Insurance plc UK Branch v International Energy Group Ltd* [2015]).
- ³⁶ This principle was used in *Novartis Grimsby Ltd v Cookson* [2007]—but this was not a mesothelioma case.
- ³⁷ *Sienkiewicz v Grief (UK) Ltd* [2009]. Also see *Willmore v Knowsley Metropolitan Borough Council* [2009].
- ³⁸ Section 3(1)(d) was the point of contention, as the conditions in s 3(1)(a)–(c) were clearly made out.
- ³⁹ Alongside the defendant’s appeal in *Willmore v Knowsley Metropolitan Borough Council* [2011], which was heard at the same time.
- ⁴⁰ The implication appears to be that if the evidential uncertainties disappear by future findings that fill the scientific gaps in our knowledge, the mesothelioma exception would also disappear.
- ⁴¹ This fits more closely with *McGhee v National Coal Board* [1973], upon which **Fairchild** was originally based (see **section 9.2.2.1**).
- ⁴² Reflecting further an attempt to do justice for those who contract mesothelioma due to negligent employers, the Mesothelioma Act 2014 established the Diffuse Mesothelioma Payment Scheme, which allows parties who contracted the disease after 25 July 2012, but who have no one to sue (as the employer(s) who negligently exposed them, or their insurers, have become insolvent), to claim from a compensation fund paid for by levies from the insurance industry (though only for 80 per cent of the amount of the average damages obtainable via legal action).
- ⁴³ See the opinion of Lord Mance, especially at [45]–[49].

- ^{44.} *Heneghan v Manchester Dry Docks Ltd and others* [2016]. Part of the reasoning was based on the fact that in *Zurich*, Lords Neuberger and Reed said that the **Fairchild** exception was ‘applicable to any disease which has the unusual features of mesothelioma’ (at [191]). Note, however, that the **Barker** rule on apportionment would apply.
- ^{45.} Compare *Meiklejohn v St George’s Healthcare NHS Trust and another* [2014].
- ^{46.} See *Chester v Afshar* [2002] (CA) (at [40]).
- ^{47.} *Wallace v Kam* [2013].
- ^{48.} See also *Thefaut v Johnston* [2017]; *Webster (A Child) v Burton Hospitals NHS Foundation Trust* [2017].
- ^{49.} See also *Pomphrey v Secretary of State for Health and another* [2019]. *Khan v Meadows* [2021] and the ‘scope of duty’ is discussed further in **section 2.5.4**.
- ^{50.} Notably, this case was decided before **Wilsher** reached the House of Lords, as recognised by Lord Bingham in **Fairchild** at [28].
- ^{51.} See also *Gouldsmith v Mid-Staffordshire General Hospitals Trust* [2007], *Ganz v Childs and others* [2011], *Less & Carter v Hussain* [2012] and *Palmer v Portsmouth Hospitals NHS Trust* [2017].
- ^{52.} Applying **Hotson**, Sriya’s claim in the scenarios outlined at the beginning of the chapter would also fail.
- ^{53.} The difference is that in **Barnett** the claimant was 100 per cent certain to die from the original arsenic poisoning: he literally had ‘no chance’.
- ^{54.} See e.g. *Allied Maples Group v Simmons and Simmons* [1995], *Dixon v Clement Jones Solicitors* [2004], *Wellesley Partners LLP v Withers LLP* [2015] (in relation to concurrent liability in contract and tort), *McGill v The Sports and Entertainment Media Group and others* [2016]; *Wright v Lewis Silkin LLP* [2016], *Perry v Raleys Solicitors* [2019] and *King v Royal United Hospitals Bath NHS Foundation Trust* [2021] EWHC 1576 (QB).
- ^{55.} On an alternative view, the ‘loss’ that the claimant identified in **Gregg** is essentially fictional. On this view it is inaccurate to say that the claimant lost a chance—either the claimant was one of the 42 people out of 100 who would have survived with immediate treatment, or he was one of the 58 who would not. As one of the former group, the claimant had a 100 per cent chance of survival, if in the latter they had no chance. The difficulty is that we cannot know which group the claimant fell into so we are thrown back onto the statistical probability (58:42) that he fell within the latter class. Importantly, just because 42 out of 100 people survive in such a situation, it does not mean that *each one of those people* had a 42 per cent chance of survival; rather 42 of them had a 100 per cent chance of survival and 58 had no chance.
- ^{56.} However, an alternative view should again be noted, which suggests that recognising loss of chance claims would potentially change the outcome of almost every negligence claim. As discussed earlier, as things stand, if a claimant can establish on the balance of probabilities that the defendant’s negligence caused their loss, they recover all of their losses. Where claimants cannot prove this, they recover nothing. However, very rarely are we 100 per cent sure that the claimant would have been harmed even without the breach, so almost every claimant could argue that they lost some chance of escaping unharmed. Moreover, where negligence claims succeed, we are rarely 100 per cent certain that the harm would not have been suffered but for the breach, because the question is necessarily hypothetical. Would acceptance of the loss of chance argument entail reductions in claimants’ damages to reflect the fact that there was a chance that the harm would have been suffered anyway? The lost chance approach involves a shift away from the all or nothing approach we currently have. If so, cases like **Hotson**, **Wilsher** and even **Fairchild** (where 100 per cent

of the loss was recoverable despite the uncertainties) would have to be overruled, unless we could say that, *in some situations*, the all or nothing approach applies, *whereas in others*, the lost chance approach does. This would be difficult to do without arbitrariness and inconsistency, though Lord Nicholls in his dissent sought to do just this.

⁵⁷ This is simply a question of apportionment. See e.g. ***Rothwell v Chemical and Insulating Co Ltd* [2007]**.

⁵⁸ Opinions differ as to whether ***Gregg*** rules out this type of claim entirely or merely restricts it to as yet undefined situations (Lord Phillips and, possibly, Baroness Hale refused to rule out lost chance claims in other contexts). See Lord Neuberger MR in *Wright (A Child) v Cambridge Medical Group* [2011] at [82]–[84].

⁵⁹ One tenuous way of putting this is that the decision could be justified on the ground that the claimant would not have been working in the scrap-metal yard (so would not have been shot) had it not been for the negligence of the first defendant. For a more concrete exposition of this idea (though rejected), see ***Gray v Thames Trains* [2009]**, where ***Baker*** was doubted, and also *Ecila Henderson (A Protected Party, by her litigation friend, The Official Solicitor) v Dorset Healthcare University NHS Foundation Trust* [2020] UKSC (these cases are considered further in the context of the defence of illegality in **section 10.3**).

⁶⁰ Though see Lord Brown, ***Gray v Thames Trains* [2009]** at [96]–[97]. Cf *Corr v IBC Vehicles Ltd* [2008].

⁶¹ In *Performance Cars v Abraham* [1962], the same idea is used in a case concerning property damage. See also *Reaney v University Hospital of North Staffordshire NHS Trust and another* [2015], discussed in **section 9.3.1.1**.

⁶² And it has recently been confirmed by the Supreme Court that the burden of proving that a loss is too remote falls on defendants: *Armstead v Royal & Sun Alliance Insurance Co Ltd* [2024].

⁶³ By the time of *The Wagon Mound (No 2)*, evidence showed that though *unlikely*, it was quite *possible* for oil spread thinly on the surface of water to be ignited if a certain temperature was reached, such as if a welder's spark fell onto something floating in the oily water and smouldered for a while (as had, in fact, happened in this case). Therefore, the issue in that case was not about foreseeability but whether, given that the damage *could* now be considered foreseeable, a reasonable person would have taken steps to prevent it—a breach issue (**section 8.5**).

⁶⁴ See e.g. **section 19.3.1**. Jenny Steele, however, says that it is perhaps 'time to update' ***The Wagon Mound (No 1)*** test (*Tort Law* (4th edn OUP 2017) 209).

⁶⁵ Confirmed in *Conarken Group Ltd v Network Rail Infrastructure Ltd* [2011], *Taylor v Chief Constable of Hampshire Police* [2013], *Hicks v Young* [2015] and by the Court of Appeal in *Goldscheider v Royal Opera House Covent Garden Foundation* [2019]. In the context of financial losses, see *Rubenstein v HSBC Bank plc* [2012].

⁶⁶ Mark Lunney, Donal Nolan and Ken Oliphant *Tort Law: Text and Materials* (6th edn OUP 2017) 280.

⁶⁷ See **section 5.5**.

⁶⁸ *Reaney v University Hospital of North Staffordshire NHS Trust and another* [2015]. The matter was reverted to a further trial applying that test: see *Reaney v University Hospital of North Staffordshire NHS Trust and another (No 2)* [2016].

⁶⁹ This obviously has links to the situations in which a *duty* in respect of the acts of a third party can arise (see **Chapter 4**, but contrast *Topp v London Country Bus (South West) Ltd* [1993], **section 4.5.3**).

⁷⁰ Contrast *Wilkin-Shaw v Fuller* [2013], where the actions of the intervening third party regarding a party of schoolchildren in difficulty on a moor were at the very least well-meaning.

- ⁷¹ Unless it is *considerably* negligent or ‘egregious’: see Lord Neuberger MR’s discussion of this in *Wright (A Child) v Cambridge Medical Group* [2011] at [37].
- ⁷² Contrast to *Rouse v Squires* [1973].
- ⁷³ Consider this in relation to Babatunde’s claim against David. Did the police break the chain of causation? Compare *Knightley v Johns* [1982], *Rouse v Squires* [1973] and *Wright v Lodge* [1993]. Also see *Robinson v Post Office* [1974].
- ⁷⁴ See **section 10.4**.
- ⁷⁵ Law Reform (Contributory Negligence) Act 1945.
- ⁷⁶ *Borealis AB v Geogas Trading SA* [2010], applied in *Flanagan and another v Greenbanks Ltd (t/a Lazenby Insulation) and Cross* [2013].
- ⁷⁷ See also *Smith v Youth Justice Board for England and Wales and another* [2010] where the claimant’s own contribution to the post-traumatic stress disorder she suffered after the death of a 15-year-old boy in custody who she was involved in forcibly restraining in effect broke the chain of causation (on grounds of ‘fairness’): see Sedley LJ at [29]–[37].

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